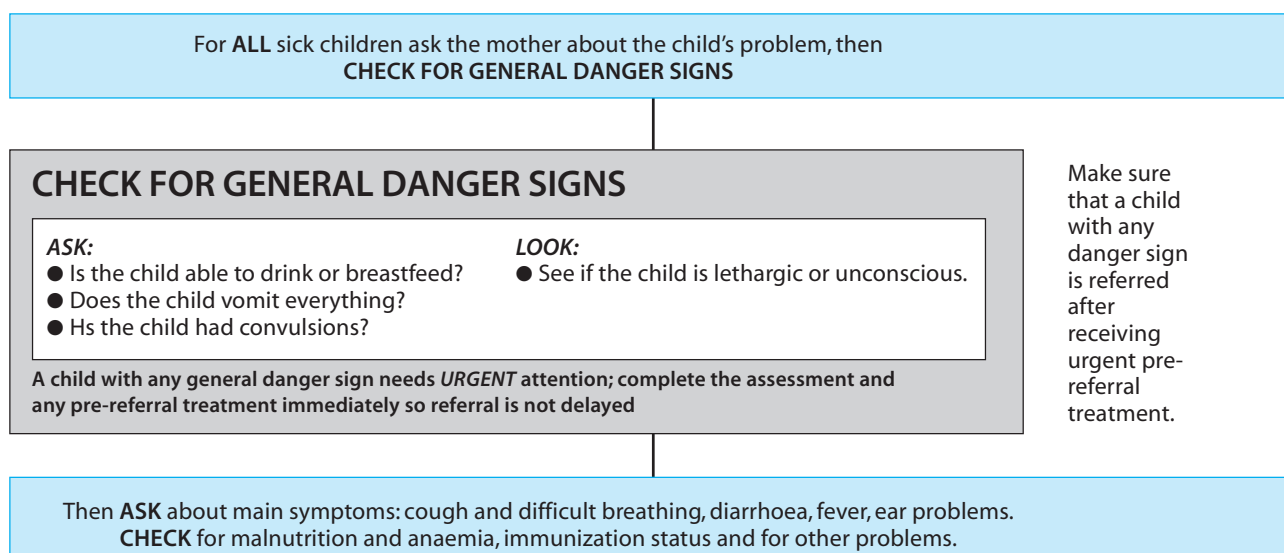


CHAPTER 6

General danger signs



Moving down the left side of the *ASSESS AND CLASSIFY* chart, you find a box titled **CHECK FOR GENERAL DANGER SIGNS**. Ask the questions and look for the clinical signs described in this box.

A child with a general danger sign has a serious problem. Most children with a general danger sign need **URGENT** referral to hospital. They may need lifesaving treatment with injectable antibiotics, oxygen or other treatments that may not be available in a first-level health facility. Complete the rest of the assessment immediately. Urgent pre-referral treatments are described in **Chapters 17 and 20**.

When you check for general danger signs:

▼ **ASK: IS THE CHILD ABLE TO DRINK OR BREASTFEED?**

A child has the sign “not able to drink or breastfeed” if the child is not able to suck or swallow when offered a drink or breastmilk.

When you ask the mother if the child is able to drink, make sure that she understands the question. If she says that the child is not able to drink or breastfeed, ask her to describe what happens when she offers the child something to drink. For example, is the child able to take fluid into his mouth and swallow it? If you are not sure about the mother's answer, ask her to offer the child a drink of clean water or breastmilk. Look to see if the child is swallowing the water or breastmilk.

A child who is breastfed may have difficulty sucking when his nose is blocked. If the child's nose is blocked, clear it. If the child can breastfeed after the nose is cleared, the child does not have the danger sign, “not able to drink or breastfeed.”

▼ **ASK: DOES THE CHILD VOMIT EVERYTHING?**

A child who is not able to hold anything down at all has the sign “vomits everything.” What goes down comes back up. A child who vomits everything will not be able to hold

down food, fluids or oral drugs. A child who vomits several times but can hold down some fluids does not have this general danger sign.

When you ask the question, use words the mother understands. Give her time to answer. If the mother is not sure if the child is vomiting everything, help her to make her answer clear. For example, ask the mother how often the child vomits. Also ask if each time the child swallows food or fluids, does the child vomit? If you are not sure of the mother's answers, ask her to offer the child a drink. See if the child vomits.

▼ ASK: HAS THE CHILD HAD CONVULSIONS?

During a convulsion, the child's arms and legs stiffen because the muscles are contracting. The child may lose consciousness or not be able to respond to spoken directions. Ask the mother if the child has had convulsions **during this current illness**. Use words the mother understands. For example, the mother may know convulsions as "fits" or "spasms."

▼ LOOK TO SEE IF THE CHILD IS LETHARGIC OR UNCONSCIOUS.

A lethargic child is not awake and alert when she should be. The child is drowsy and does not show interest in what is happening around him. Often the lethargic child does not look at his mother or watch your face when you talk. The child may stare blankly and appear not to notice what is going on around him. An unconscious child cannot be wakened. He does not respond when he is touched, shaken or spoken to.

Ask the mother if the child seems unusually sleepy or if she cannot wake the child. Look to see if the child awakens when the mother talks or shakes the child or when you clap your hands.

Note: If the child is sleeping and has cough or difficult breathing, count the number of breaths first before you try to wake the child (see **Chapter 7**).

On the case recording form, circle any general danger signs that are found, and check (✓) the appropriate answer in the CLASSIFY column (see Example 3).

EXAMPLE 3: TOP PART OF A RECORDING FORM WITH GENERAL DANGER SIGNS

MANAGEMENT OF THE SICK CHILD AGE 2 MONTHS UP TO 5 YEARS			
Name: <u>Fatima</u>	Age: <u>18 months</u>	Weight: <u>11.5</u> kg	Temperature: <u>37.5</u> °C
ASK: What are the child's problems? <u>cough, trouble breathing</u>		Initial visit? ☒	Follow-up Visit? ☐
ASSESS (Circle all signs present)		CLASSIFY	
CHECK FOR GENERAL DANGER SIGNS NOT ABLE TO DRINK OR BREASTFEED VOMITS EVERYTHING CONVULSIONS		General danger signs present? Yes ☒ No ☐ Remember to use danger sign when selecting classifications	

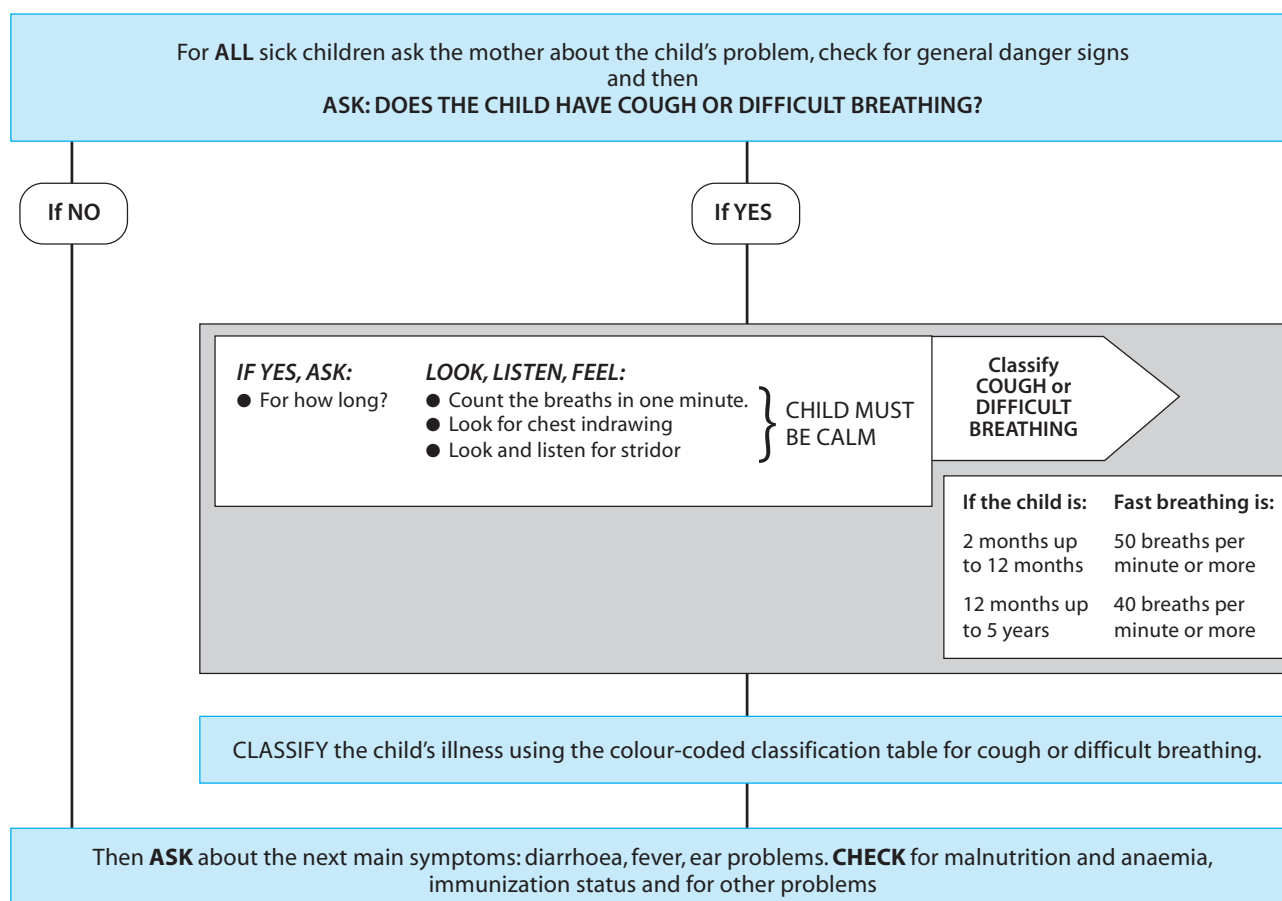
CASE 1: Fatima is 18 months old. She weighs 11.5 kg. Her temperature is 37.5 °C. The health worker asked, "What are the child's problems?" The mother said "Fatima has been coughing for 6 days, and she is having trouble breathing." This is the initial visit for this illness.

The health worker checked Fatima for general danger signs. The mother said that Fatima is able to drink. She has not been vomiting. She has not had convulsions during this illness. The health worker asked, "Does Fatima seem unusually sleepy?" The mother said, "Yes." The health worker clapped his hands. He asked the mother to shake the child. Fatima opened her eyes, but did not look around. The health worker talked to Fatima, but she did not watch his face. She stared blankly and appeared not to notice what was going on around her.

IF THE CHILD HAS A GENERAL DANGER SIGN, COMPLETE THE REST OF THE ASSESSMENT IMMEDIATELY. THIS CHILD HAS A SEVERE PROBLEM. THERE MUST BE NO DELAY IN HIS OR HER TREATMENT.

CHAPTER 7

Cough or difficult breathing



Respiratory infections can occur in any part of the respiratory tract such as the nose, throat, larynx, trachea, air passages or lungs. A child with cough or difficult breathing may have pneumonia or another severe respiratory infection. Pneumonia is an infection of the lungs. Both bacteria and viruses can cause pneumonia. In developing countries, pneumonia is often due to bacteria. The most common are *Streptococcus pneumoniae* and *Hemophilus influenzae*. Children with bacterial pneumonia may die from hypoxia (too little oxygen) or sepsis (generalized infection).

Many children are brought to the clinic with less serious respiratory infections. Most children with cough or difficult breathing have only a mild infection. For example, a child who has a cold may cough because nasal discharge drips down the back of the throat. Or the child may have a viral infection of the bronchi called bronchitis. These children are not seriously ill. They do not need treatment with antibiotics. Their families can manage them at home.

You need to identify the few, very sick children with cough or difficult breathing who need treatment with antibiotics. Fortunately, you can identify almost all cases of pneumonia by checking for these two clinical signs: fast breathing and chest indrawing.

CHAPTER 15

Assess and classify the sick young infant

This chapter describes the steps to assess and classify a sick young infant during an initial visit. The steps are:

- Check for signs of possible bacterial infection. Then classify the young infant based on the clinical signs found.
- Ask about diarrhoea. If the infant has diarrhoea, assess for related signs. Classify the young infant for dehydration. Also classify for persistent diarrhoea and dysentery if present.
- Check for feeding problem or low weight. This may include assessing breastfeeding. Then classify feeding.
- Check the young infant's immunization status.
- Assess any other problems.

If you find a reason that a young infant needs urgent referral, you should continue the assessment. However, skip the breastfeeding assessment because it can take some time.

15.1 How to check a young infant for possible bacterial infection

For ALL sick young infants check for signs of POSSIBLE BACTERIAL INFECTION

CHECK FOR POSSIBLE BACTERIAL INFECTION

ASK:

- Has the infant had convulsions?

LOOK, LISTEN, FEEL:

- Count the breaths in one minute. Repeat the count if elevated.
- Look for severe chest indrawing.
- Look for nasal flaring.
- Look and listen for grunting.
- Look and feel for bulging fontanelle.
- Look for pus draining from the ear.
- Look at the umbilicus. Is it red or draining pus? Does the redness extend to the skin?
- Measure temperature (or feel for fever or low body temperature).
- Look for skin pustules. Are there many or severe pustules?
- See if the young infant is lethargic or unconscious.
- Look at the young infant's movements. Are they less than normal?

} YOUNG INFANT MUST BE CALM

CLASSIFY the infant's illness using the colour-coded classification table for possible bacterial infection.

Then ASK about diarrhoea. CHECK for feeding problem or low weight, immunization status and for other problems.

This assessment step is done for **every** sick young infant. In this step you are looking for signs of bacterial infection, especially a serious infection. A young infant can become sick and die very quickly from serious bacterial infections such as pneumonia, sepsis and meningitis.

It is important to assess the signs in the order on the chart, and to keep the young infant calm. The young infant must be calm and may be asleep while you assess the first four signs, that is, count breathing and look for chest indrawing, nasal flaring and grunting.

To assess the next few signs, you will pick up the infant and then undress him, look at the skin all over his body and measure his temperature. By this time he will probably be awake. Then you can see if he is lethargic or unconscious and observe his movements.

Check for possible bacterial infection in **ALL** young infants.

▼ **ASK: HAS THE INFANT HAD CONVULSIONS?**

Ask the mother this question.

▼ **LOOK: COUNT THE BREATHS IN ONE MINUTE. REPEAT THE COUNT IF ELEVATED**

Count the breathing rate as you would in an older infant or young child. Young infants usually breathe faster than older infants and young children. The breathing rate of a healthy young infant is commonly more than 50 breaths per minute. Therefore, 60 breaths per minute or more is the cutoff used to identify fast breathing in a young infant.

If the first count is 60 breaths or more, repeat the count. This is important because the breathing rate of a young infant is often irregular. The young infant will occasionally stop breathing for a few seconds, followed by a period of faster breathing. If the second count is also 60 breaths or more, the young infant has fast breathing.

▼ **LOOK FOR SEVERE CHEST INDRAWING**

Look for chest indrawing as you would look for chest indrawing in an older infant or young child. However, mild chest indrawing is normal in a young infant because the chest wall is soft. Severe chest indrawing is very deep and easy to see. Severe chest indrawing is a sign of pneumonia and is serious in a young infant.

▼ **LOOK FOR NASAL FLARING**

Nasal flaring is widening of the nostrils when the young infant breathes in.



Normal position of nostrils



Nostrils flare when infant breathes in

▼ LOOK AND LISTEN FOR GRUNTING

Grunting is the soft, short sounds a young infant makes when breathing out. Grunting occurs when an infant is having trouble breathing.

▼ LOOK AND FEEL FOR BULGING FONTANELLE

The fontanelle is the soft spot on the top of the young infant's head, where the bones of the head have not formed completely. Hold the young infant in an upright position. The infant must not be crying. Then look at and feel the fontanelle. If the fontanelle is bulging rather than flat, this may mean the young infant has meningitis.

▼ LOOK FOR PUS DRAINING FROM THE EAR

Pus draining from the ear is a sign of infection. Look inside the infant's ear to see if pus is draining from the ear.

▼ LOOK AT THE UMBILICUS—IS IT RED OR DRAINING PUS? DOES THE REDNESS EXTEND TO THE SKIN?

There may be some redness of the end of the umbilicus or the umbilicus may be draining pus. (The cord usually drops from the umbilicus by one week of age.) How far down the umbilicus the redness extends determines the severity of the infection. If the redness extends to the skin of the abdominal wall, it is a serious infection.

▼ FEEL: MEASURE TEMPERATURE (OR FEEL FOR FEVER OR LOW BODY TEMPERATURE)

Fever (axillary temperature more than 37.5 °C or rectal temperature more than 38 °C) is uncommon in the first two months of life. If a young infant has fever, this may mean the infant has a serious bacterial infection. In addition, fever may be the only sign of a serious bacterial infection. Young infants can also respond to infection by dropping their body temperature to below 35.5 °C (36 °C rectal temperature). Low body temperature is called hypothermia. If you do not have a thermometer, feel the infant's stomach or axilla (underarm) and determine if it feels hot or unusually cool.

▼ LOOK FOR SKIN PUSTULES. ARE THERE MANY OR SEVERE PUSTULES?

Examine the skin on the entire body. Skin pustules are red spots or blisters that contain pus. If you see pustules, is it just a few pustules or are there many? A severe pustule is large or has redness extending beyond the pustule. Many or severe pustules indicate a serious infection.

▼ LOOK: SEE IF THE YOUNG INFANT IS LETHARGIC OR UNCONSCIOUS

Young infants often sleep most of the time, and this is not a sign of illness. Even when awake, a healthy young infant will usually not watch his mother and a health worker while they talk, as an older infant or young child would.

A lethargic young infant is not awake and alert when he should be. He may be drowsy and may not stay awake after a disturbance. If a young infant does not wake up during the assessment, ask the mother to wake him. Look to see if the child awakens when the mother talks or gently shakes the child or when you clap your hands. See if he stays awake.

An unconscious young infant cannot be wakened at all. He does not respond when he is touched or spoken to.

▼ LOOK AT THE YOUNG INFANT'S MOVEMENTS. ARE THEY LESS THAN NORMAL?

A young infant who is awake will normally move his arms or legs or turn his head several times in a minute if you watch him closely. Observe the infant's movements while you do the assessment.

15.2 How to classify possible bacterial infection

Classify all sick young infants for bacterial infection. Compare the infant's signs to signs listed on the colour-coded table and choose the appropriate classification. There are two possible classifications for bacterial infection: **POSSIBLE SERIOUS BACTERIAL INFECTION** and **LOCAL BACTERIAL INFECTION** (see Example 19). In **Parts IV, V and VI** you will read about how to identify treatments and to treat young infants with these classifications.

EXAMPLE 19: CLASSIFICATION TABLE FOR POSSIBLE BACTERIAL INFECTION

SIGNS	CLASSIFY AS	IDENTIFY TREATMENT (Urgent pre-referral treatments are in bold print.)
● Convulsions or ● Fast breathing (60 breaths per minute or more) or ● Severe chest indrawing or ● Nasal flaring or ● Grunting or ● Bulging fontanelle or ● Pus draining from ear or ● Umbilical redness extending to the skin or ● Fever (37.5 °C* or above or feels hot) or low body temperature (less than 35.5 °C* or feels cold) or ● Many or severe skin pustules or ● Lethargic or unconscious or ● Less than normal movement.	POSSIBLE SERIOUS BACTERIAL INFECTION	➤ Give first dose of intramuscular antibiotics. ➤ Treat to prevent low blood sugar. ➤ Advise mother how to keep the infant warm on the way to hospital. ➤ Refer URGENTLY to hospital
● Red umbilicus or draining pus or ● Skin pustules.	LOCAL BACTERIAL INFECTION	➤ Give an appropriate oral antibiotic. ➤ Teach the mother to treat local infections at home. ➤ Advise mother to give home care for the young infant. ➤ Follow-up in 2 days.

* These thresholds are based on axillary temperature. The thresholds for rectal temperature readings are approximately 0.5 °C higher.

POSSIBLE SERIOUS BACTERIAL INFECTION

A young infant with signs in this classification may have a serious disease and be at high risk of dying. The infant may have pneumonia, sepsis or meningitis. It is difficult to distinguish between these infections in a young infant. Fortunately, it is not necessary to make this distinction.

A young infant with any sign of **POSSIBLE SERIOUS BACTERIAL INFECTION** needs urgent referral to hospital. Before referral, give a first dose of intramuscular antibiotics and treat to prevent low blood sugar. Malaria is unusual in infants of this age, so give no treatment for possible severe malaria. Advise the mother to keep her sick young infant warm. Young infants have difficulty maintaining their body temperature. Low temperature alone can kill young infants.

LOCAL BACTERIAL INFECTION

Young infants with this classification have an infected umbilicus or a skin infection.

Treatment includes giving an appropriate oral antibiotic at home for 5 days. You will learn more about how to treat the infant and counsel the mother in later chapters.

15.3 How to assess and classify a young infant for diarrhoea

For **ALL** sick young infants check for signs of possible bacterial infection and then
ASK: DOES THE YOUNG INFANT HAVE DIARRHOEA?

IF YES: ASSES AND CLASSIFY the young infant's diarrhoea using the DIARRHOEA box in the *YOUNG INFANT* chart. The process is very similar to the one used for the sick child (see Chapter 8).

Then **CHECK** for feeding problem or low weight, immunization status and other problems.

If the mother says that the young infant has diarrhoea, assess and classify for diarrhoea. The normally frequent or loose stools of a breastfed baby are not diarrhoea. The mother of a breastfed baby can recognize diarrhoea because the consistency or frequency of the stools is different than normal. The assessment is similar to the assessment of diarrhoea for an older infant or young child, but ***fewer signs are checked***. Thirst is not assessed. This is because it is not possible to distinguish thirst from hunger in a young infant.

Diarrhoea in a young infant is ***classified in the same way*** as in an older infant or young child. Compare the infant's signs to the signs listed and choose one classification for dehydration. Choose an additional classification if the infant has diarrhoea for 14 days or more, or has blood in the stool.

Note: *there is only one possible classification for persistent diarrhoea in a young infant. This is because any young infant who has persistent diarrhoea has suffered with diarrhoea a large part of his life and should be referred.*

15.4 How to check a young infant for feeding problem or low weight

Adequate feeding is essential for growth and development. Poor feeding during infancy can have lifelong effects. Growth is assessed by determining weight for age. It is important to assess a young infant's feeding and weight so that feeding can be improved if necessary.

The best way to feed a young infant is to breastfeed exclusively. Exclusive breastfeeding means that the infant takes only breastmilk, and no additional food, water or other fluids. (Medicines and vitamins are exceptions.)

Exclusive breastfeeding gives a young infant the best nutrition and protection from disease possible. If mothers understand that *exclusive* breastfeeding gives the best chances of good growth and development, they may be more willing to breastfeed. They may be motivated to breastfeed to give their infants a good start in spite of social or personal reasons that make exclusive breastfeeding difficult or undesirable.

The assessment has two parts. In the first part, you ask the mother questions. You determine if she is having difficulty feeding the infant, what the young infant is fed and how often. You also determine weight for age.

In the second part, if the infant has any problems with breastfeeding or is low weight for age, you assess how the infant breastfeeds.

For **ALL** sick young infants check for signs of possible bacterial infection, ask about diarrhoea and then **CHECK FOR FEEDING PROBLEM OR LOW WEIGHT**.

THEN CHECK FOR FEEDING PROBLEM OR LOW WEIGHT

ASK:

- Is there any difficulty feeding?
- Is the infant breastfed? If yes, how many times in 24 hours?
- Does the infant usually receive any other foods or drinks? If yes, how often?
- What do you use to feed the infant?

LOOK, LISTEN, FEEL:

- Determine weight for age.

IF AN INFANT:

**Has any difficulty feeding,
Is breastfeeding less than 8 times in 24 hours,
Is taking any other foods or drinks, or
Is low weight for age,
AND
Has no indications to refer urgently to hospital:**

ASSESS BREASTFEEDING:

- Has the infant breastfed in the previous hour?

If the infant has not fed in the previous hour, ask the mother to put her infant to the breast. Observe the breastfeed for 4 minutes.

(If the infant was fed during the last hour, ask the mother if she can wait and tell you when the infant is willing to feed again.)

- Is the infant able to attach?
no attachment at all not well attached good attachment

TO CHECK ATTACHMENT, LOOK FOR:

- Chin touching breast
- Mouth wide open
- Lower lip turned outward
- More areola visible above than below the mouth

(All these signs should be present if the attachment is good.)

- Is the infant suckling effectively (that is, slow deep sucks, sometimes pausing)?
no suckling at all not suckling effectively suckling effectively

Clear a blocked nose if it interferes with breastfeeding.

- Look for ulcers or white patches in the mouth (thrush).

CLASSIFY the infant's nutritional status using the colour-coded classification table for feeding problem or low weight.

Then **CHECK** immunization status and for other problems.

15.4.1 How to ask about feeding and determine weight for age

▼ ASK: IS THERE ANY DIFFICULTY FEEDING?

Any difficulty mentioned by the mother is important. This mother may need counselling or specific help with a difficulty.¹ If a mother says that the infant is **not able to feed**,

¹ Breastfeeding difficulties mentioned by a mother may include: her infant feeds too frequently, or not frequently enough; she does not have enough milk; her nipples are sore; she has flat or inverted nipples; or the infant does not want to take the breast.

assess breastfeeding or watch her try to feed the infant with a cup to see what she means by this. An infant who is **not able to feed** may have a serious infection or other life-threatening problem and should be referred urgently to hospital.

▼ **ASK: IS THE INFANT BREASTFED? IF YES, HOW MANY TIMES IN 24 HOURS?**

The recommendation is that the young infant be breastfed as often and for as long as the infant wants, day and night. This should be 8 or more times in 24 hours.

▼ **ASK: DOES THE INFANT USUALLY RECEIVE ANY OTHER FOODS OR DRINKS? IF YES, HOW OFTEN?**

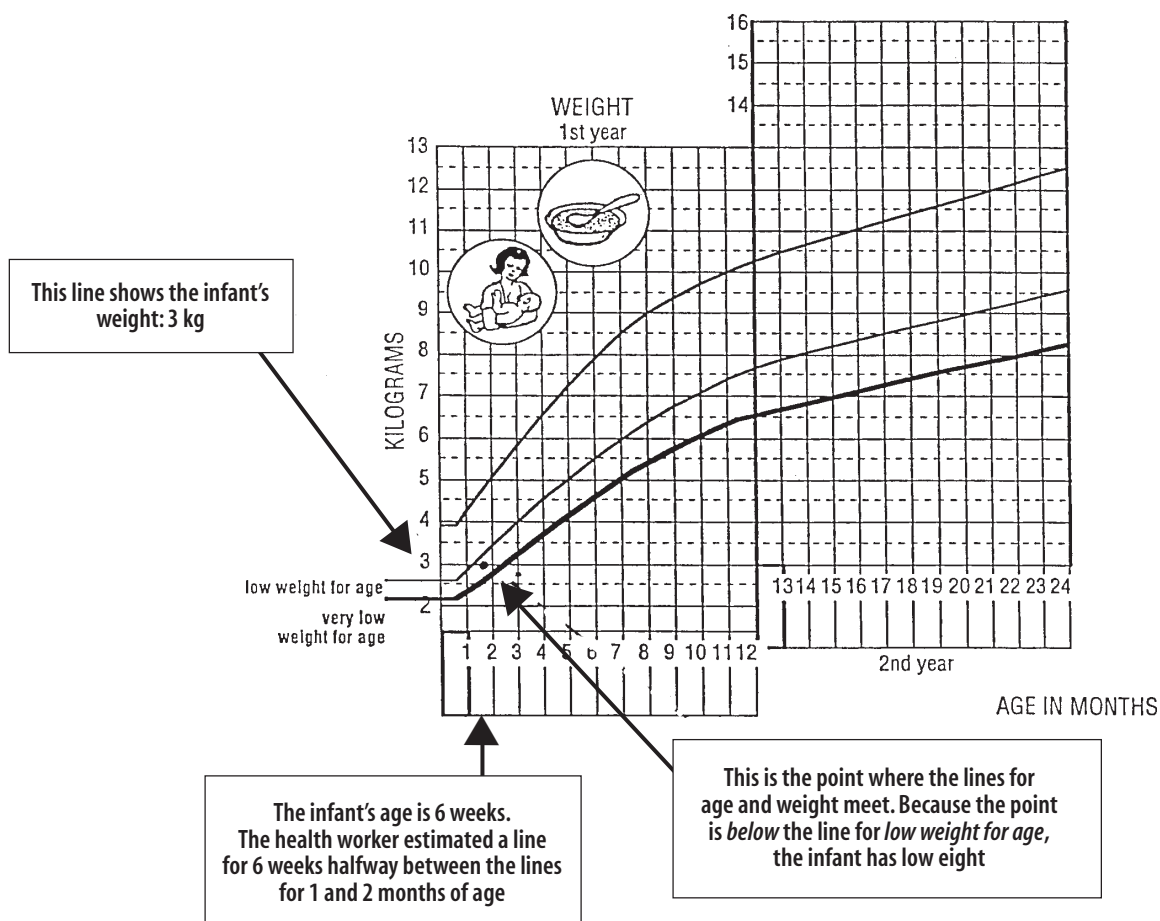
A young infant should be exclusively breastfed. Find out if the young infant is receiving **any** other foods or drinks such as other milk, juice, tea, thin porridge, dilute cereal, or even water. Ask how often he receives it and the amount. You need to know if the infant is mostly breastfed, or mostly fed on other foods.

▼ **ASK: WHAT DO YOU USE TO FEED THE INFANT?**

If an infant takes other foods or drinks, find out if the mother uses a feeding bottle or cup.

▼ **LOOK: DETERMINE WEIGHT FOR AGE**

Use a weight for age chart to determine if the young infant is low weight for age. Notice that **for a young infant you should use the Low Weight for Age line**, instead of the Very Low Weight for Age line, which is used for older infants and children. **Remember that the age of a young infant is usually stated in weeks, but the Weight for Age chart is labeled in months.** Some young infants who are low weight for age were born with low birthweight. Some did not gain weight well after birth.



15.4.2 How to assess breastfeeding

First decide whether to assess the infant's breastfeeding:

- If the infant is exclusively breastfed without difficulty and is not low weight for age, there is no need to assess breastfeeding.
- If the infant is not breastfed at all, do not assess breastfeeding.
- If the infant has a serious problem requiring urgent referral to a hospital, do not assess breastfeeding.

In these situations, classify the feeding based on the information that you have already.

If the mother's answers or the infant's weight indicates a difficulty, observe a breastfeed as described below. Low weight for age is often due to low birthweight. Low birthweight infants are particularly likely to have a problem with breastfeeding. Assessing breastfeeding requires careful observation.

▼ ASK: HAS THE INFANT BREASTFED IN THE PREVIOUS HOUR?

If yes, ask the mother to wait and tell you when the infant is willing to feed again. In the meantime, complete the assessment by assessing the infant's immunization status. You may also decide to begin any treatment that the infant needs, such as giving an antibiotic for LOCAL BACTERIAL INFECTION or ORS solution for SOME DEHYDRATION.

If the infant has not fed in the previous hour, he may be willing to breastfeed. Ask the mother to put her infant to the breast. Observe a whole breastfeed if possible, or observe for at least 4 minutes. Sit quietly and watch the infant breastfeed.

▼ LOOK: IS THE INFANT ABLE TO ATTACH?

The four signs of good attachment are (If all of these four signs are present, the infant has **good attachment**):

- chin touching breast (or very close)
- mouth wide open
- lower lip turned outward
- more areola visible above than below the mouth

If attachment is not good, you may see (If you see any of these signs of poor attachment, the infant is **not well attached**):

- chin not touching breast
- mouth not wide open, lips pushed forward
- lower lip turned in, or
- more areola (or equal amount) visible below infant's mouth than above it

If a very sick infant cannot take the nipple into his mouth and keep it there to suck, he



A baby well attached to his mother's breast



A baby poorly attached to his mother's breast

has ***no attachment at all***. He is not able to breastfeed at all. If an infant is not well attached, the results may be pain and damage to the nipples. Or the infant may not remove breastmilk effectively, which may cause engorgement of the breast. The infant may be unsatisfied after breastfeeds and want to feed very often or for a very long time. The infant may get too little milk and not gain weight, or the breastmilk may dry up. All these problems may improve if attachment can be improved.

▼ LOOK: IS THE INFANT SUCKLING EFFECTIVELY? (THAT IS, SLOW DEEP SUCKS, SOMETIMES PAUSING)

The infant is ***suckling effectively*** if he suckles with slow deep sucks and sometimes pauses. You may see or hear the infant swallowing. If you can observe how the breastfeed finishes, look for signs that the infant is satisfied. If satisfied, the infant releases the breast spontaneously (that is, the mother does not cause the infant to stop breastfeeding in any way). The infant appears relaxed, sleepy, and loses interest in the breast.

An infant is ***not suckling effectively*** if he is taking only rapid, shallow sucks. You may also see indrawing of the cheeks. You do not see or hear swallowing. The infant is not satisfied at the end of the feed, and may be restless. He may cry or try to suckle again, or continue to breastfeed for a long time.

An infant who is ***not suckling at all*** is not able to suck breastmilk into his mouth and swallow. Therefore he is not able to breastfeed at all. If a blocked nose seems to interfere with breastfeeding, clear the infant's nose. Then check whether the infant can suckle more effectively.

▼ LOOK FOR ULCERS OR WHITE PATCHES IN THE MOUTH (THRUSH)

Look inside the mouth at the tongue and inside of the cheek. Thrush looks like milk curds on the inside of the cheek, or a thick white coating of the tongue. Try to wipe the white off. The white patches of thrush will remain.

15.5 How to classify feeding problem or low weight

Compare the young infant's signs to the signs listed in each row of the colour-coded classification table and choose the appropriate classification. There are three possible classifications for feeding problem or low weight: NOT ABLE TO FEED—POSSIBLE SERIOUS BACTERIAL INFECTION, FEEDING PROBLEM OR LOW WEIGHT, and NO FEEDING PROBLEM (see Example 20).

In **Parts IV, V and VI** you will read about how to identify treatments and to treat young infants with these classifications.

NOT ABLE TO FEED—POSSIBLE SERIOUS BACTERIAL INFECTION

The young infant who is not able to feed has a life-threatening problem. This could be due to a bacterial infection or another sort of problem.¹ The infant requires immediate attention. Treatment is the same as for the classification POSSIBLE SERIOUS BACTERIAL INFECTION at the top of the chart. Refer the young infant urgently to hospital.

FEEDING PROBLEM OR LOW WEIGHT

This classification includes infants who are low weight for age or infants who have some sign that their feeding needs improvement. They are likely to have more than one of these signs. Advise the mother of any young infant in this classification to breastfeed as often and for as long as the infant wants, day and night. Short breastfeeds are an impor-

¹ An infant with neonatal tetanus who has stopped being able to feed and has stiffness would be referred based on this classification.

EXAMPLE 20: CLASSIFICATION TABLE FOR FEEDING PROBLEM OR LOW WEIGHT

SIGNS	CLASSIFY AS	IDENTIFY TREATMENT (Urgent pre-referral treatments are in bold print.)
● Not able to feed or ● No attachment at all or ● Not suckling at all.	NOT ABLE TO FEED—POSSIBLE SERIOUS BACTERIAL INFECTION	➤ Give first dose of intramuscular antibiotics. ➤ Treat to prevent low blood sugar. ➤ Advise the mother how to keep the young infant warm on the way to hospital. ➤ Refer URGENTLY to hospital.
● Not well attached to breast or ● Not suckling effectively or ● Less than 8 breastfeeds in 24 hours or ● Receives other foods or drinks or ● Low weight for age or ● Thrush (ulcers or white patches in mouth).	FEEDING PROBLEM OR LOW WEIGHT	➤ Advise the mother to breastfeed as often and for as long as the infant wants, day and night. ● If not well attached or not suckling effectively, teach correct positioning and attachment. ● If breastfeeding less than 8 times in 24 hours, advise to increase frequency of feeding. ➤ If receiving other foods or drinks, counsel mother about breastfeeding more, reducing other foods or drinks, and using a cup. ● If not breastfeeding at all: — Refer for breastfeeding counselling and possible relactation. — Advise about correctly prepared breastmilk substitutes and using a cup. ➤ If thrush, teach the mother to treat thrush at home. ➤ Advise mother to give home care for the young infant. ➤ Follow-up any feeding problem or thrush in 2 days. Follow-up low weight for age in 14 days.
● Not low weight for age and no other signs of inadequate feeding.	NO FEEDING PROBLEM	➤ Advise mother to give home care for the young infant. ➤ Praise the mother for feeding the infant well.

tant reason why an infant may not get enough breastmilk. The infant should breastfeed until he is finished. Teach each mother about any specific help her infant needs, such as better positioning and attachment for breastfeeding, or treating thrush.

NO FEEDING PROBLEM

A young infant in this classification is exclusively and frequently breastfed. “Not low” weight for age means that the infant’s weight for age is not below the line for “Low Weight for Age”. It is not necessarily normal or good weight for age, but the infant is not in the high-risk category.

15.6 How to check the young infant’s immunization status

Check immunization status just as you would for an older infant or young child (see **Chapter 12**). Remember that you should not give OPV 0 to an infant who is more than 14 days old. Therefore, if an infant has *not* received OPV 0 by the time he is 15 days old, you should wait to give OPV until he or she is 6 weeks old. Then give OPV 1.

15.7 How to assess other problems

Assess any other problems mentioned by the mother or observed by you. Refer to other guidelines on treatment of those problems. If you think the infant has a serious problem, or if you do not know how to help the infant, refer the infant to hospital.

15.8 The young infant case recording form

As you assess and classify the sick young infant, circle the signs found and write the classification(s) on the young infant case recording form (see Example 21). You will find sample case recording forms in **Annex B**.

EXAMPLE 21: TOP THREE SECTIONS OF THE YOUNG INFANT CASE RECORDING FORM

MANAGEMENT OF THE SICK YOUNG INFANT AGE 1 WEEK UP TO 2 MONTHS												
Name: <u>Jomli</u> Age: <u>6 weeks</u> Weight: <u>4.5</u> kg Temperature: <u>37</u> °C ASK: What are the infant's problems? <u>diarrhoea and rash</u> Initial visit? ☒ Follow-up Visit? ☐ ASSESS (Circle all signs present) CLASSIFY												
CHECK FOR POSSIBLE BACTERIAL INFECTION - Has the infant had convulsions? - Count the breaths in one minute. <u>55</u> breaths per minute Repeat if elevated <u>Fast</u> breathing? - Look for severe chest indrawing. - Look for nasal flaring. - Look and listen for grunting. - Look and feel for bulging fontanelle. - Look for pus draining from the ear. - Look at umbilicus. Is it red or draining pus? Does the redness extend to the skin? - Fever (temperature 37.5°C or feels hot) or low body temperature (below 35.5°C or feels cool). - Look for skin pustules. Are there many or severe pustules? - See if young infant is lethargic or unconscious. - Look at young infant's movements. Less than normal?	<u>Local Bacterial Infection</u>											
DOES THE YOUNG INFANT HAVE DIARRHOEA? Yes ☒ No ☐ - For how long? <u>3</u> Days - Is there blood in the stools?	<u>Some Dehydration</u>											
THEN CHECK FOR FEEDING PROBLEM OR LOW WEIGHT - Is there any difficulty feeding? Yes ☐ No ☒ - Is the infant breastfed? Yes ☒ No ☐ - If Yes, how many times in 24 hours? <u>5</u> times - Does the infant usually receive any other foods or drinks? Yes ☒ No ☐ - If Yes, how often? <u>One bottle of cow's milk in afternoon, sometimes water also</u> - What do you use to feed the child? <u>feeding bottle</u>	<u>Feeding Problem or Low Weight</u>											
If the infant has any difficulty feeding, is feeding less than 8 times in 24 hours, is taking any other food or drinks, or is low weight for age AND has no indications to refer urgently to hospital: ASSESS BREASTFEEDING: - Has the infant breastfed in the previous hour? If infant has not fed in the previous hour, ask the mother to put her infant to the breast. Observe the breastfeed for 4 minutes. - Is the infant able to attach? To check attachment, look for: <table border="0"> <tr> <td>— Chin touching breast</td> <td>Yes ☒</td> <td>No ☐</td> </tr> <tr> <td>— Mouth wide open</td> <td>Yes ☒</td> <td>No ☐</td> </tr> <tr> <td>— Lower lip turned outward</td> <td>Yes ☒</td> <td>No ☐</td> </tr> <tr> <td>— More areola above than below the mouth</td> <td>Yes ☒</td> <td>No ☐</td> </tr> </table> no attachment at all not well attached <u>good attachment</u> - Is the infant suckling effectively (that is, slow deep sucks, sometimes pausing)? not suckling at all not suckling effectively <u>suckling effectively</u> - Look for ulcers or white patches in the mouth (thrush).		— Chin touching breast	Yes ☒	No ☐	— Mouth wide open	Yes ☒	No ☐	— Lower lip turned outward	Yes ☒	No ☐	— More areola above than below the mouth	Yes ☒
— Chin touching breast	Yes ☒	No ☐										
— Mouth wide open	Yes ☒	No ☐										
— Lower lip turned outward	Yes ☒	No ☐										
— More areola above than below the mouth	Yes ☒	No ☐										

CASE 5: Jomli is a 6-week-old infant. His weight is 4.5 kg. His axillary temperature is 37 °C. He is brought to the clinic because he has diarrhoea and a rash. It is his first visit for this illness. The health worker checks the young infant for signs of possible bacterial infection. His mother says that Jomli has not had convulsions. The health worker counts 55 breaths per minute. He finds no chest indrawing or nasal flaring. Jomli has no grunting. The fontanelle does not

bulge. There is no pus in his ears. The umbilicus is normal. The body temperature is normal. There are some skin pustules. Jomli is not lethargic or unconscious, and his movements are normal.

When the health worker asks the mother about Jomli's diarrhoea, the mother replies that it began 3 days ago, and there is no blood in the stool. Jomli is crying. He stopped once when his mother put him to the breast. He began crying again when she stopped breastfeeding. His eyes look normal, not sunken. When the skin of his abdomen is pinched, it goes back slowly.

Jomli's mother says that she has no difficulty feeding him. He breastfeeds about 5 times in 24 hours. She gives him other foods and drinks. The health worker uses the Weight for Age chart and determines that Jomli's weight (4.5 kg) is not low for his age (6 weeks).

Since Jomli is feeding less than 8 times in 24 hours and is taking other food or drinks, the health worker decides to assess breastfeeding. Jomli's mother agrees to try to breastfeed now. The health worker observes that Jomli's chin is touching the breast. His mouth is open wide, and his lower lip is turned outward. More areola is visible above than below the mouth. His sucks are deep and slow. When Jomli stops breastfeeding, the health worker looks in his mouth. He sees no ulcers or white patches in his mouth.

Part IV



IDENTIFY TREATMENT

CHAPTER 25

Use good communication skills

It is important to have good communication with the child's mother or caretaker from the beginning of the visit. Using good communication helps to reassure the mother or caretaker that the child will receive good care. A young infant or child who is treated at clinic needs to continue treatment at home. The success of home treatment depends on how well you communicate with the child's mother or caretaker. She needs to know how to give the treatment. She also needs to understand the importance of the treatment.

- **Ask** and **Listen** to find out what the child's problems are and what the mother is already doing for the child.
- **Praise** the mother for what she has done well.
- **Advise** her how to care for her child at home.
- **Check** the mother's understanding.

▼ ASK AND LISTEN TO FIND OUT WHAT THE CHILD'S PROBLEMS ARE AND WHAT THE MOTHER IS ALREADY DOING FOR HER CHILD

You have already learned in **Chapter 5** the importance of asking questions to assess the child's problems. Listen carefully to find out what the child's problems are and what the mother is already doing for her child. Then you will know what she is doing well, and what practices need to be changed.

▼ PRAISE THE MOTHER FOR WHAT SHE HAS DONE WELL

It is likely that the mother is doing something helpful for the child, for example, breastfeeding. Praise the mother for something helpful she has done. Be sure that the praise is genuine, and only praise actions that are indeed helpful to the child.

▼ ADVISE THE MOTHER HOW TO CARE FOR HER CHILD AT HOME

Limit your advice to what is relevant to the mother at this time. Use language that the mother will understand. If possible, use pictures or real objects to help explain. For example, show amounts of fluid in a cup or container.

Advise against any harmful practices that the mother may have used. When correcting a harmful practice, be clear, but also be careful not to make the mother feel guilty or incompetent. Explain why the practice is harmful.

Some advice is simple. For example, you may only need to tell the mother to return with the child for follow-up in 2 days. Other advice requires that you teach the mother how to do a task. Teaching **how to do** a task requires several steps.

Think about how you learned to write, cook or do any other task that involved special skills. You were probably first given instruction. Then you may have watched someone else. Finally you tried doing it yourself.

When you teach a mother how to treat a child, use 3 basic *teaching steps*:

1. Give **information**.
2. Show an **example**.
3. Let her **practice**.

Give information: Explain to the mother how to do the task. For example, explain to the mother how to:

- apply eye ointment,
- prepare ORS, or
- soothe a sore throat.

Show an example: Show how to do the task. For example, show the mother:

- how to hold a child still and apply eye ointment,
- a packet of ORS and how to mix the right amount of water with ORS, or
- a safe remedy to soothe the throat which she could make at home.

Let her practice: Ask the mother to do the task while you watch. For example, have the mother:

- apply eye ointment in her child's eye,
- mix ORS solution, or
- describe how she will prepare a safe remedy to soothe the throat.

It may be enough to ask the mother to describe how she will do the task at home.

Letting a mother **practise** is the most important part of teaching a task. If a mother **does** a task while you observe, you will know what she understands and what is difficult. You can then help her do it better. The mother is more likely to remember something that she has **practised** than something that she has heard.

When teaching the mother:

- Use words that she understands.
- Use teaching aids that are familiar, such as common containers for mixing ORS solution.
- Give feedback when she practices. Praise what was done well and make corrections.
- Allow more practice, if needed.
- Encourage the mother to ask questions. Answer all questions.

▼ CHECK THE MOTHER'S UNDERSTANDING

Ask questions to find out what the mother understands and what needs further explanation. Avoid asking leading questions (that is, questions which suggest the right answer) and questions that can be answered with a simple yes or no.

Examples of good checking questions are: “What foods will you give your child?” “How often will you give them?” If you get an unclear response, ask another checking question. Praise the mother for correct understanding or clarify your advice as necessary.

After you teach a mother how to treat her child, you want to be sure that she understands how to give the treatment correctly. Checking questions find out what a mother has learned.

An important communication skill is knowing how to ask good checking questions. A checking question must be phrased so that the mother answers more than “yes” or “no”. Good checking questions require that she describe **why**, **how** or **when** she will give a treatment.

From her answer you can tell if she has understood you and learned what you taught her about the treatment. If she cannot answer correctly, give more information or clarify your instructions. For example, you taught a mother how to give an antibiotic. Then you ask:

“Do you know how to give your child his medicine?”

The mother would probably answer “yes” whether she understands or not. She may be embarrassed to say she does not understand. However, if you ask a few good checking questions, such as:

“When will you give your child the medicine?”

“How many tablets will you give each time?”

“For how many days will you give the tablets?”

you are asking the mother to repeat back to you instructions that you have given her. Asking good checking questions helps you make sure that the mother learns and remembers how to treat her child.

The following questions check a mother’s understanding. “Good checking questions” require the mother to describe **how** she will treat her child. They begin with question words, such as **why, what, how, when, how many**, and **how much**. The “poor questions”, answered with a “yes” or “no”, do not show you how much a mother knows.

After you ask a question, pause. Give the mother a chance to think and then answer. Do **not** answer the question for her. Do **not** quickly ask a different question.

Asking checking questions requires patience. The mother may know the answer, but she may be slow to speak. She may be surprised that you really expect her to answer. She may fear her answer will be wrong. She may feel shy to talk to an authority figure. Wait for her to answer. Give her encouragement.

GOOD CHECKING QUESTIONS	POOR QUESTIONS
How will you prepare the ORS solution?	Do you remember how to mix the ORS?
How often should you breastfeed your child?	Should you breastfeed your child?
On what part of the eye do you apply the ointment?	Have you used ointment on your child before?
How much extra fluid will you give after each loose stool?	Do you know how to give extra fluids?
Why is it important for you to wash your hands?	Will you remember to wash your hands?

If the mother answers incorrectly or says she does not remember, be careful not to make her feel uncomfortable. Teach her to give the treatment again. Give more **information, examples** or **practice** to make sure she understands. Then ask her good checking questions again.

A mother may understand but may say that she cannot do as you ask. She may have a problem or objection. Common problems are lack of time or resources to give the treatment. A mother may object that her sick child was given an oral drug rather than an injection, or a home remedy rather than a drug.

Help the mother think of possible solutions to her problems and respond to her objections. For example:

If you ask,

“When will you apply the eye ointment in your child’s eye?”

The mother may answer that she is not at home during the day. She may tell you that she can only treat her child in the morning and in the night.

Ask her if she can identify someone (a grandparent, an older sibling) who will be at home during the day and can give the mid-day treatment. Help her plan how she will teach that person to give the treatment correctly.

If you ask,

“What container will you use to measure 1 litre of water for mixing ORS?”

The mother may answer that she does not have a 1-litre container at home.

Ask her what containers she does have at home. Show her how to measure 1 litre of water in her container. Explain how to mark the container at 1 litre with an appropriate tool or how to measure 1 litre using several smaller containers.

If you ask,

“How will you soothe your child’s throat at home?”

A mother may answer that she does not like the remedy that you recommended. She expected her child to get an injection or tablets instead.

Convince her of the importance of the safe remedy rather than the drug. Make the explanation clear. She may have to explain the reason for the safe remedy to family members who also expected the child to be treated differently.

When checking the mother’s understanding:

- Ask questions that require the mother to explain what, how, how much, how many, when, or why. Do **not** ask questions that can be answered with just a “yes” or “no”.
- Give the mother time to think and then answer.
- Praise the mother for correct answers.
- If she needs it, give more **information, examples** or **practice**.

CHAPTER 26

Teach the caretaker to give oral drugs at home

The oral drugs listed on the TREAT charts are given for different reasons, in different doses and on different schedules. However, the way to give each drug is similar. This chapter will give you the basic steps for teaching mothers to give oral drugs. If a mother learns how to give a drug correctly, then the child will be treated properly. Follow the instructions below for every oral drug you give to the mother.

▼ DETERMINE THE APPROPRIATE DRUGS AND DOSAGE FOR THE CHILD'S AGE OR WEIGHT.

Use the *TREAT THE CHILD* chart to determine the appropriate drug and dosage to give the child. Use the *YOUNG INFANT* chart to determine the appropriate drug and dosage for young infants.

▼ TELL THE MOTHER THE REASON FOR GIVING THE DRUG TO THE CHILD, INCLUDING:

- why you are giving the oral drug to her child, and
- what problem it is treating.

▼ DEMONSTRATE HOW TO MEASURE A DOSE.

Collect a container of the drug and check its expiry date. Do not use expired drugs. Count out the amount needed for the child. Close the container.

If you are giving the mother **tablets**:

Show the mother the amount to give per dose. If needed, show her how to divide a tablet. If a tablet has to be crushed before it is given to a child, add a few drops of clean water and wait a minute or so. The water will soften the tablet and make it easier to crush.

If you are giving the mother **syrup**:

Show the mother how to measure the correct number of millilitres (ml) for one dose at home. Use the bottle cap or a common spoon, such as a spoon used to stir sugar into tea or coffee. Show her how to measure the correct dose with the spoon.

One teaspoon (tsp.) equals approximately 5.0 ml (see below).

MILLILITRES (ml)	TEASPOONS (tsp.)
1.25 ml	$\frac{1}{4}$ tsp.
2.5 ml	$\frac{1}{2}$ tsp.
5.0 ml	1 tsp.
7.5 ml	$1\frac{1}{2}$ tsp.
10.0 ml	2 tsp.
15.0 ml	3 tsp.

Adjust the above amounts based on the common spoons in your area.

If you are giving the mother **capsules**:

Show the mother the amount to give per dose. If a child needs less than a whole vitamin A capsule (or cannot swallow a whole capsule), show the mother how to open the capsule and squirt part of its liquid into the child's mouth.

▼ **WATCH THE MOTHER PRACTICE MEASURING A DOSE BY HERSELF.**

Ask the mother to measure a dose by herself. If the dose is in tablet form and the child cannot swallow a tablet, tell the mother to crush the tablet. Watch her as she practices. Tell her what she has done correctly. If she measured the dose incorrectly, show her again how to measure it.

▼ **ASK THE MOTHER TO GIVE THE FIRST DOSE TO HER CHILD.**

Explain that if a child is vomiting, give the drug even though the child may vomit it up. Tell the mother to watch the child for 30 minutes. If the child vomits within the 30 minutes (the tablet or syrup may be seen in the vomit), give another dose. If the child is dehydrated and vomiting, wait until the child is rehydrated before giving the dose again.

▼ **EXPLAIN CAREFULLY HOW TO GIVE THE DRUG, THEN LABEL AND PACKAGE THE DRUG.**

Tell the mother how much of the drug to give her child. Tell her how many times per day to give the dose. Tell her when to give it (such as early morning, lunch, dinner, before going to bed) and for how many days.

Write the information on a drug label. This is an example.

NAME:		DATE:	
DRUG:		QUANTITY:	
		 	
DOSE:			

Follow the steps below:

- Write the full name of the drug and the total amount of tablets, capsules or syrup to complete the course of treatment.
- Write the correct dose for the patient to take (number of tablets, capsules, squirts or spoonfuls, that is, $\frac{1}{2}$, 1, $1\frac{1}{2}$...). Write when to give the dose (early morning, lunch, dinner, before going to bed).
- Write the daily dose and schedule, such as

$\frac{1}{2}$ tablet twice daily for 5 days

Write the instructions clearly so that a literate person is able to read and understand them. Put the total amount of each drug into its own labelled drug container (an envelope, paper, tube or bottle). Keep drugs clean. Use clean containers. After you have labelled and packaged the drug, give it to the mother. Ask checking questions to make sure she understands how to treat her child.

▼ **IF MORE THAN ONE DRUG WILL BE GIVEN, COLLECT, COUNT AND PACKAGE EACH DRUG SEPARATELY.**

Collect one drug at a time. Write the instructions on the label. Count out the amount needed. Put enough of the drug into its own labelled package. Finish packaging the drug before you open another drug container.

Explain to the mother that her child is getting more than one drug because he has more than one illness. Show the mother the different drugs. Explain how to give each drug. If necessary, draw a summary of the drugs and times to give each drug during the day.

▼ **EXPLAIN THAT ALL THE ORAL DRUG TABLETS OR SYRUPS MUST BE USED TO FINISH THE COURSE OF TREATMENT, EVEN IF THE CHILD GETS BETTER.**

Explain to the mother that if the child seems better, continue to treat the child. This is important because the bacteria or the malaria parasite may still be present even though the signs of disease are gone.

Advise the mother to keep all medicines out of the reach of children. Also tell her to store drugs in a dry and dark place that is free of mice and insects.

▼ **CHECK THE MOTHER'S UNDERSTANDING BEFORE SHE LEAVES THE CLINIC.**

Ask the mother checking questions, such as:

“How much will you give each time?”

“When will you give it?” “For how many days?”

“How will you prepare this tablet?”

“Which drug will you give 3 times per day?”

If you feel that the mother is likely to have problems when she gives her child the drug(s) at home, offer more **information, examples** and **practice**. A child needs to be treated correctly to get better.

In some clinics, a drug dispenser has the task of teaching the mother to give treatment and checking the mother's understanding. If this is your situation, teach the skills you are learning in this section to that dispenser.

CHAPTER 27

Teach the caretaker to treat local infections at home

This chapter will describe how to teach a child's mother or caretaker to treat local infections at home. Local infections include cough, sore throat, eye infection, mouth ulcers, ear infection, an umbilicus that is red or draining pus, skin pustules, and thrush.

When teaching a mother or caretaker:

- Explain what the treatment is and why it should be given.
- Describe the treatment steps listed in the appropriate box on the TREAT charts.
- Watch the mother as she does the first treatment in the clinic (except remedy for cough or sore throat).
- Tell her how often to do the treatment at home.
- If needed for treatment at home, give mother the tube of tetracycline ointment or a small bottle of gentian violet.
- Check the mother's understanding before she leaves the clinic.

Some treatments for local infections cause discomfort. Children often resist having their eyes, ears or mouth treated. Therefore, it is important to hold the child still. This will prevent the child from interfering with the treatment.

The drawing on the right shows a good position for holding a child. Tilt the child's head back when applying eye ointment or treating mouth ulcers. Tilt the child's head to the side when wicking the ear.



Do **not** attempt to hold the child still until immediately before treatment.

27.1 Treat eye infection with tetracycline eye ointment

If the child will be **URGENTLY** referred, clean the eye gently. Pull down the lower lid. Squirt the first dose of tetracycline eye ointment onto the lower eyelid. The dose is about the size of a grain of rice.

If the child is not being referred, teach the mother to apply the tetracycline eye ointment. Refer to the *TREAT THE CHILD* chart and give the mother the following information.

Tell the mother that she should treat both eyes to prevent damage to the eyes. Tell her also that the ointment will slightly sting the child's eye. Tell her to:

- Wash her hands before and after treating the eye.
- Clean the child's eyes immediately before applying the tetracycline eye ointment. Use a clean cloth to wipe the eye.

- Repeat the process (cleaning the eye and applying ointment) 3 times per day, in the morning, at mid-day and in the evening.

Then **show the mother** how to treat the eye. Be sure to wash your hands.

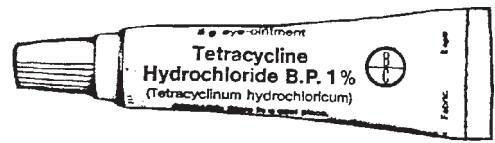
- Hold down the lower lid of your eye. Point to the lower lid. Tell the mother that this is where she should apply the ointment. Tell her to be careful that the tube does not touch the eye or lid.
- Have someone hold the child still.
- Wipe one of the child's eyes with the cloth. Squirt the ointment onto the lower lid. Make sure the mother sees where to apply the ointment and the amount (the size of a grain of rice).

Ask the mother to **practise** cleaning and applying the eye ointment into the child's other eye. Observe and give feedback as she practices. When she is finished, give her the following additional information.

- **Treat both eyes until the redness is gone from the infected eye.** The infected eye is improving if there is less pus in the eye or the eyes are not stuck shut in the morning.
- Do **not** put any other eye ointments, drops or alternative treatments in the child's eyes. They may be harmful and damage the child's eyes. Putting harmful substances in the eye may cause blindness.
- After 2 days, if there is still pus in the eye, bring the child back to the clinic.

Then give the mother the tube of ointment to take home. Give her the same tube you used to treat the child in the clinic. Before the mother leaves, ask **checking questions**. Check that she understands how to treat the eye. For example, ask:

- “Will you treat one or both eyes?”
- “How much ointment you will put in the eyes? Show me.”
- “How often will you treat the eyes?”
- “When will you wash your hands?”



27.2 Dry the ear by wicking

Refer to the *TREAT THE CHILD* chart. To teach a mother how to dry the ear by wicking, first **tell her** it is important to keep an infected ear dry to allow it to heal. Then **show her** how to wick her child's ear.

As you wick the child's ear, tell the mother to:

- Use clean, absorbent cotton cloth or soft strong tissue paper for making a wick. Do **not** use a cotton-tipped applicator, a stick or flimsy paper that will fall apart in the ear.
- Place the wick in the child's ear until the wick is wet.
- Replace the wet wick with a clean one.
- Repeat these steps until the wick stays dry. Then the ear is dry.



Observe the mother as she practises. Give feedback. When she is finished, give her the following information.

- Wick the ear 3 times daily.
- Use this treatment **for as many days as it takes** until the wick no longer gets wet when put in the ear and no pus drains from the ear.
- Do **not** place anything (oil, fluid, or other substance) in the ear between wicking treatments. Do **not** allow the child to go swimming. No water should get in the ear.

Ask checking questions, such as:

- “What materials will you use to make the wick at home?”
 “How many times per day will you dry the ear with a wick?”
 “What else will you put in your child’s ear?”

If the mother thinks she will have problems wicking the ear, help her solve them.

27.3 Treat mouth ulcers with gentian violet

Treating mouth ulcers controls infection and helps the child to eat. Teach the mother to treat mouth ulcers with *half-strength* gentian violet. Gentian violet used in the mouth should be **half-strength** (0.25%), not full-strength (0.5%). Give the following **information**. Tell the mother:

- Her child will start eating normally sooner if she paints the mouth ulcers in her child’s mouth. It is important that the child eats.
- Clean the child’s mouth. Wrap a clean soft cloth around her finger. Dip it in salt water. Wipe the mouth.
- Use a clean cloth or a cotton-tipped stick to paint gentian violet on the mouth ulcers. The gentian violet will kill germs that cause the ulcers. Put a small amount of gentian violet on the cloth or stick. Do **not** let the child drink the gentian violet.
- Treat the mouth ulcers 2 times per day, in the morning and evening.
- **Treat the mouth ulcers for 5 days and then stop.**

Wrap a clean cloth around your finger and dip it into salt water. **Show the mother** how to first wipe the child’s mouth clean. Then paint half of the child’s mouth with half-strength gentian violet.

Ask the mother to practise. Watch her wipe the child’s mouth clean and paint the rest of the ulcers with gentian violet. Comment on the steps she did well and those that need to be improved.

Give the mother a bottle of *half-strength* gentian violet to take home. Tell her to return in 2 days for follow-up. Also tell her that she should return to the clinic earlier if the mouth ulcers get worse or if the child is not able to drink or eat.

Before the mother leaves, **ask checking questions**. For example, ask:

- “What will you use to clean the child’s mouth?”
 “When will you wash your hands?”
 “How often will you treat the child’s mouth?” “For how many days?”

If she anticipates any problems providing the treatment, help her to solve them.

27.4 Soothe the throat, relieve the cough with a safe remedy

To soothe the throat or relieve a cough, use a safe remedy. Such remedies can be home-made, given at the clinic, or bought at a pharmacy. It is important that they are safe. Home-made remedies are as effective as those bought in a store.

Your *TREAT THE CHILD* chart recommends safe, soothing remedies for children with a sore throat or cough. If the child is exclusively breastfed, do **not** give other drinks or remedies. Breastmilk is the best soothing remedy for an exclusively breastfed child. When explaining how to give the safe remedy, it is not necessary to watch the mother practice giving the remedy to the child. Exact dosing is not important with this treatment.

Harmful remedies may be used in your area. If so, they have been recorded in the box. Never use remedies that contain harmful ingredients, such as atropine, codeine or codeine derivatives, or alcohol. These items may sedate the child. They may interfere with the child’s feeding. They may also interfere with the child’s ability to cough up secretions from the lungs. Medicated nose drops (that is, nose drops that contain anything other than salt) should also not be used.

27.5 Treat the young infant for local infections

There are three types of local infections in a young infant that a mother or caretaker can treat at home: an umbilicus that is red or draining pus, skin pustules, and thrush. These local infections are treated in the same way that mouth ulcers are treated in an older infant or child. Follow the instructions in the TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER section of the *YOUNG INFANT* chart. Twice each day, the mother cleans the infected area and then applies gentian violet. Teach the mother how to treat the infection and check her understanding. If the mother will treat thrush, give her a bottle of **half-strength** (0.25%) gentian violet. If the mother will treat skin pustules or umbilical infection, give her a bottle of full strength (0.5%) gentian violet.

CHAPTER 28

Counsel the mother about breastfeeding problems

In **Chapter 15** you learned to check a young infant for feeding problem or low weight. If the infant is breastfeeding and was classified as **FEEDING PROBLEM OR LOW WEIGHT**, you need to counsel the mother of the infant about any breastfeeding problems that were found during the assessment.

- If a mother is breastfeeding her infant less than 8 times in 24 hours, advise her to increase the frequency of breastfeeding. Breastfeed as often and for as long as the infant wants, day and night.
- If the infant receives other foods or drinks, counsel the mother about breastfeeding more, reducing the amount of the other foods or drinks, and if possible, stopping altogether. Advise her to feed the infant any other drinks from a cup, and not from a feeding bottle.
- If the mother does not breastfeed at all, consider referring her for breastfeeding counselling and possible relactation. If the mother is interested, a breastfeeding counselor may be able to help her to overcome difficulties and begin breastfeeding again.

Advise a mother who does not breastfeed about choosing and correctly preparing an appropriate breastmilk substitute (see **Chapter 29 Counsel the Mother About Feeding and Fluids**). Also advise her to feed the young infant with a cup, and not from a feeding bottle.

Follow-up any young infant with a feeding problem in 2 days. This is especially important if you are recommending a significant change in the way that the infant is fed.

28.1 Teach correct positioning and attachment for breastfeeding

In **Chapter 15** you also learned to assess breastfeeding if an infant does *not* need **URGENT** referral to hospital and:

- has any difficulty feeding,
- is breastfeeding less than 8 times in 24 hours,
- is taking any other foods or drinks, or
- is low weight for age,

If you observe problems with attachment or suckling during the breastfeed, you need to teach the infant's mother about correct positioning and attachment.

28.1.1 Reasons for Poor Attachment and Ineffective Suckling

There are several reasons that an infant may be poorly attached or not able to suckle effectively. He may have had bottle feeds, especially in the first few days after delivery. His mother may be inexperienced. She may have had some difficulty and nobody to help or advise her. For example, perhaps the infant was small and weak, the mother's nipples were flat or there was a delay starting to breastfeed.

CHAPTER 29

Counsel the mother about feeding and fluids

For many sick infants and children, you will need to assess feeding and counsel the mother about feeding and fluids. Recommendations on FOOD, FLUID and WHEN TO RETURN are given on the *COUNSEL THE MOTHER* chart (referred to in this chapter as the *COUNSEL* chart). Additional recommendations for the young infant are given in the TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER section of the *YOUNG INFANT* chart.

29.1 Feeding recommendations

Feeding Recommendations During Sickness and Health are given on the *COUNSEL* chart. The recommendations are listed in columns for different age groups. You need to understand all of the feeding recommendations, but you will not need to explain them all to any one mother. You will first ask questions to find out how her child is already being fed. Then you will give **only the advice that is needed** for the child's age and situation.

These feeding recommendations are appropriate both when the child is sick and when the child is healthy. During illness, children may not want to eat much. However, they should be offered the types of food recommended for their age, as often as recommended, even though they may not take much at each feeding. After illness, good feeding helps make up for weight loss and helps prevent malnutrition. When the child is well, good feeding helps prevent future illness.

Sick child visits are a good opportunity to counsel the mother on how to feed the child both during illness and when the child is well.

29.1.1 Recommendations for ages up to 4 months

The best way to feed a child from birth to at least 4 months of age is to breastfeed exclusively. Exclusive breastfeeding means that the child takes only breastmilk and no additional food, water, or other fluids (with the exception of medicines and vitamins, if needed). Breastfeed children at this age as often as they want, day and night. This will be at least 8 times in 24 hours.

The advantages of breastfeeding are described below:

Breastmilk contains exactly the nutrients needed by an infant. It contains: Protein, Fat, Lactose (a special milk sugar), Vitamins A, C and Iron

These nutrients are more easily absorbed from breastmilk than from other milk. Breastmilk also contains essential fatty acids needed for the infant's growing brain, eyes, and blood vessels. These fatty acids are not available in other milks.

Breastmilk provides all the water an infant needs, even in a hot, dry climate.

Breastmilk protects an infant against infection. An infant cannot fight infection as well as an older child or an adult. Through breastmilk, an infant can share his mother's ability to fight infection. Exclusively breastfed infants are less likely to get diarrhoea, and less likely to die from diarrhoea or other infections. Breastfed infants are less likely to develop pneumonia, meningitis, and ear infections than non-breastfed infants are.

Breastfeeding helps a mother and baby to develop a close, loving relationship.

Breastfeeding protects a mother's health. After delivery, breastfeeding helps the uterus return to its previous size. This helps reduce bleeding and prevent anaemia. Breastfeeding also reduces the mother's risk of ovarian cancer and breast cancer.

It is best not to give an infant any milk or food other than breastmilk. For example, do not give cow's milk, goat's milk, formula, cereal, or extra drinks such as teas, juices, or water.

Reasons:

- Giving other food or fluid reduces the amount of breastmilk taken.
- Other food or fluid may contain germs from water or on feeding bottles or utensils. These germs can cause infection.
- Other food or fluid may be too dilute, so that the infant becomes malnourished.
- Other food or fluid may not contain enough Vitamin A.
- Iron is poorly absorbed from cow's and goat's milk.
- The infant may develop allergies.
- The infant may have difficulty digesting animal milk, so that the milk causes diarrhoea, rashes, or other symptoms. Diarrhoea may become persistent.

Exclusive breastfeeding will give an infant the best chance to grow and stay healthy.

29.1.2 Recommendations for ages 4 months up to 6 months

Most babies do not need complementary foods before 6 months of age. Breastmilk remains the child's most important food, but at some time between the ages of 4 and 6 months, some children begin to need foods in addition to breastmilk. These foods are often called complementary or weaning foods because they complement breastmilk.

The mother should only begin to offer complementary foods if the child shows interest in semisolid foods, appears hungry after breastfeeding, or is not gaining weight adequately. The child may show interest by reaching for the mother's food, or by opening her mouth eagerly when food is offered.

By 6 months of age, all children should be receiving a thick, nutritious complementary food. It is important to continue to breastfeed as often as the child wants, day and night. The mother should give the complementary foods 1–2 times daily **after** breastfeeding to avoid replacing breastmilk.

29.1.3 Recommendations for ages 6 months up to 12 months

The mother should continue to breastfeed as often as the child wants. However, after 6 months of age, breastmilk cannot meet all of the child's energy needs. From age 6 months up to 12 months, gradually increase the amount of complementary foods given. Foods that are appropriate in your country are listed on the COUNSEL chart. By the age of 12 months, complementary foods are the main source of energy.

If the child is breastfed, give complementary foods 3 times daily. If the child is not breastfed, give complementary foods 5 times daily. (If possible, include feedings of milk by cup. However, cow's milk and other breastmilk substitutes are not as good for babies as breastmilk.)

It is important to actively feed the child. Active feeding means encouraging the child to eat. The child should not have to compete with older brothers and sisters for food from a common plate. He should have his own serving. Until the child can feed himself, the mother or another caretaker (such as an older sibling, father, or grandmother) should sit with the child during meals and help get the spoon into his mouth.

An "adequate serving" means that the child does not want any more food after active feeding.

Good complementary foods

Good complementary foods are energy-rich, nutrient-rich, and locally affordable. Examples in some areas are thick cereal with added oil or milk, fruits, vegetables, pulses, meat, eggs, fish, and milk products. If the child receives cow's milk or any other breastmilk substitute, these and any other drinks should be given by cup, *not* by bottle. Foods that are appropriate in your area are listed in the *Feeding Recommendations* box of the *COUNSEL* chart.

29.1.4 Recommendations for ages 12 months up to 2 years

During this period the mother should continue to breastfeed as often as the child wants and also give nutritious complementary foods. The variety and quantity of food should be increased. Family foods should become an important part of the child's diet. Family foods should be chopped so that they are easy for the child to eat.

Give nutritious complementary foods or family foods 5 times a day. Adequate servings and active feeding (encouraging the child to eat) continue to be important.

29.1.5 Recommendations for ages 2 years and older

At this age the child should be taking a variety of family foods in 3 meals per day. The child should also be given 2 extra feedings per day. These may be family foods or other nutritious foods that are convenient to give between meals. Examples are listed on the chart.

29.2 Special recommendations for children with persistent diarrhoea

The *COUNSEL* chart lists some special feeding recommendations for children with persistent diarrhoea. Children with persistent diarrhoea may have difficulty digesting milk other than breastmilk. They need to temporarily reduce the amount of other milk in their diet. They must take more breastmilk or other foods to make up for this reduction.

Tell the mother:

■ **If still breastfeeding**, give more frequent, longer breastfeeds, day and night.

■ **If taking other milk:**

- replace with increased breastfeeding OR
- replace with fermented milk products, such as yoghurt OR
- replace half the milk with nutrient-rich semisolid food.

■ For other foods, follow feeding recommendations for the child's age.

The child with persistent diarrhoea should be seen again in 5 days for follow-up.

29.3 Assess the child's feeding

You need to assess feeding of children who:

- are classified as having ANAEMIA OR VERY LOW WEIGHT, or
- are less than 2 years old.

However, if the mother has already received many treatment instructions and is overwhelmed, you may delay assessing feeding and counselling the mother about feeding until a later visit.

Even though you may feel hurried, it is important to take time to counsel the mother carefully and completely. When counselling a mother about feeding, you will use some of the same communication skills described in **Chapter 25**.

For example, you will ask the mother questions to determine how she is feeding the child. You will *listen carefully* to the mother's answers so that you can make your advice relevant to her. You will praise the mother for appropriate practices and advise her about any practices that need to be changed. You will use simple language that the mother can understand. Finally, you will ask *checking questions* to ensure that the mother knows how to care for her child at home.

To assess feeding, ask the mother the following questions. These questions are at the top of the *COUNSEL* chart and also at the bottom of the sick child case recording form. These questions will help you find out about the child's usual feeding and feeding during this illness:

- *Do you breastfeed your child?*
If yes: *how many times during the day?*
Do you also breastfeed during the night?
- *Does the child take any other food or fluids?*
If yes: *What food or fluids?*
How many times per day?
What do you use to feed the child?
If very low weight for age: *How large are servings? Does the child receive his own serving?*
Who feeds the child and how?
- *During this illness, has the child's feeding changed? If yes, how?*

Note that certain questions are asked only if the child is very low weight for age. For these children, it is important to take the extra time to ask about serving size and active feeding.

Listen for correct feeding practices as well as those that need to be changed. As you listen to the mother, you may look at the *Feeding Recommendations During Sickness and Health* that are appropriate for the child's age on the *COUNSEL* chart. If an answer is unclear, ask another question. For example, if the mother of a very-low-weight child says that servings are "large enough," you could ask, "When the child has eaten, does he still want more?"

29.4 Identify feeding problems

It is important to complete the assessment of feeding and identify all the feeding problems **before** giving advice. Based on the mother's answers to the feeding questions, identify any differences between the child's actual feeding and the recommendations on the *COUNSEL* chart. These differences are problems. Some examples of feeding problems are listed below.

EXAMPLES OF FEEDING PROBLEMS

CHILD'S ACTUAL FEEDING	RECOMMENDED FEEDING
A 3-month-old is given sugar water as well as breastmilk.	A 3-month-old should be given only breastmilk and no other food or fluid.
A 2-year-old is fed only 3 times each day.	A 2-year-old should receive 2 extra feedings between meals, as well as 3 meals a day.
An 8-month-old is still exclusively breastfed.	A breastfed 8-month-old should also be given adequate servings of a nutritious complementary food 3 times a day.

In addition to differences from the feeding recommendations, some other problems may become apparent from the mother's answers. Common problems are listed on the *Counsel the Mother About Feeding Problems* section of the *COUNSEL* chart. Examples of such problems are:

Difficulty breastfeeding

The mother may mention that breastfeeding is uncomfortable for her, or that her child seems to have difficulty breastfeeding. If so, you will need to assess breastfeeding as described on the *YOUNG INFANT* chart. You may find that the infant's positioning and attachment could be improved.

Use of feeding bottle

Feeding bottles should not be used. They are often dirty, and germs easily grow in them. Fluids tend to be left in them and soon become spoiled or sour. The child may drink the spoiled fluid and become ill. Also, sucking on a bottle may interfere with the child's desire to breastfeed.

Lack of active feeding

Young children often need to be encouraged and assisted to eat. This is especially true if a child has very low weight. If a young child is left to feed himself, or if he has to compete with siblings for food, he may not get enough to eat. By asking, "Who feeds the child and how?" you should be able to find out if the child is actively being encouraged to eat.

Not feeding well during illness

The child may be eating much less, or eating different foods during illness. Children often lose their appetite during illness. However, they should still be encouraged to eat the types of food recommended for their age, as often as recommended, even if they do not eat much. They should be offered their favourite nutritious foods, if possible, to encourage eating.

As you assess a child's feeding, note the mother's answers and write any feeding problems found in the *ASSESS CHILD'S FEEDING* section of the case recording form (see Example 26).

EXAMPLE 26: PART OF THE CASE RECORDING FORM FOR A 4-MONTH-OLD CHILD WITH THE CLASSIFICATION NO ANAEMIA AND NOT VERY LOW WEIGHT

ASSESS (Circle all signs present)	CLASSIFY
ASSESS CHILD'S FEEDING if child has ANAEMIA OR VERY LOW WEIGHT or is less than 2 years old - Do you breastfeed your child? Yes ☒ No ☐ - If Yes, how many times in 24 hours? <u>5</u> times. Do you breastfeed during the night? Yes ☒ No ☐ - Does the child take any other food or fluids? Yes ☒ No ☐ - If Yes, what food or fluids? <u>Cow's milk</u> - How many times per day? <u>3</u> times. What do you use to feed the child? <u>Feeding bottle</u> - If very low weight for age: How large are servings? _____ - Does the child receive how own serving? _____ Who feeds the child and how? _____ - During the illness, has the child's feeding changed? Yes ☐ No ☒ - If Yes, how? _____	FEEDING PROBLEMS <i>Not breastfed often enough</i> <i>Giving Cow's milk</i> <i>Using feeding bottle</i>

29.5 Counsel the mother about feeding problems

Since you have identified feeding problems, you will be able to limit your advice to what is most relevant to the mother. When counselling a mother about feeding problems, it is important to use good communications skills (see **Chapter 25**). Remember to:

- **Ask and Listen** to find out what the child's feeding problems are and what the mother is already doing for the child.
- **Praise** the mother for what she has done well.
- **Advise** her how to feed her child. Limit your advice to what is relevant to the mother at this time.
- **Check** the mother's understanding.

If the feeding recommendations are being followed and there are no problems, praise the mother for her good feeding practices. Encourage her to keep feeding the child the same way during illness and health. If the child is about to enter a new age group with different feeding recommendations, explain these new recommendations to her. For example, if the child is almost 6 months old, explain about good complementary foods and when to start them.

If the feeding recommendations for the child's age are not being followed, explain those recommendations. In addition, if you found any of the problems listed on the *COUNSEL* chart in the section *Counsel the Mother About Feeding Problems*, give the mother the recommended advice:

- **If the mother reports difficulty with breastfeeding, assess breastfeeding. As needed, show the mother correct positioning and attachment for breastfeeding. (See the *YOUNG INFANT* chart and Chapter 28)**

You learned to check and improve breastfeeding positioning and attachment in **Chapters 15 and 28**. If the mother has a breast problem, such as engorgement, sore nipples, or a breast infection, then she may need referral to a specially trained breastfeeding counsellor (such as a health worker who has taken *Breastfeeding Counselling: A Training Course*) or to someone experienced in managing breastfeeding problems, such as a midwife.

- **If the child is less than 4 months old and is taking other milk or foods:**

If a child under 4 months old is receiving food or fluids other than breastmilk, the goal is to gradually change back to more or exclusive breastfeeding. Suggest giving more frequent, longer breastfeeds, day and night. As breastfeeding increases, the mother should gradually reduce other milk or food. Since this is an important change in the child's feeding, be sure to ask the mother to return for follow-up in 5 days.

In some cases, changing to more or exclusive breastfeeding may be impossible (for example, if the mother never breastfed, if she must be away from her child for long periods, or if she will not breastfeed for personal reasons). In such cases, the mother should be sure to correctly prepare cow's milk or other breastmilk substitutes and use them within an hour to avoid spoilage. It is important to use the correct amount of clean, boiled water for dilution.

To prepare cow's milk for infants less than 3 months of age, mix $\frac{1}{2}$ cup boiled whole cow's milk with $\frac{1}{2}$ cup boiled water and 2 level teaspoons of sugar. Each level teaspoon of sugar should equal 5 grams. A cup contains 200 ml. Adjust the recipe if you have different size cups or teaspoons

- **If the mother is using a bottle to feed the child:**

A cup is better than a bottle. A cup is easier to keep clean and does not interfere with breastfeeding. To feed a baby by cup:

- Hold the baby sitting upright or semi-upright on your lap.
- Hold a small cup to the baby's lips. Tip the cup so the liquid just reaches the baby's lips.

- The baby becomes alert and opens his mouth and eyes.
- A low-birthweight baby takes the milk into his mouth with the tongue.
- A full-term or older baby sucks the milk, spilling some of it.
- Do not **pour** the milk into the baby's mouth. Just hold the cup to his lips and let him take it himself.
- When the baby has had enough, he closes his mouth and will not take more.

► **If the child is not being fed actively:**

Counsel the mother to sit with the child and encourage eating. Tell her to give the child an adequate serving in a separate plate or bowl.

► **If the child is not feeding well during illness:**

Even though children often lose their appetites during illness, they should be encouraged to eat the types of food recommended for their age, as often as recommended. Offer the child's favourite nutritious foods to encourage eating. Offer small feedings frequently. After illness, good feeding helps make up for any weight loss and prevent malnutrition.

Note the feeding advice that you give on the reverse side of the case recording form.

29.5 Use a mother's card

A Mother's Card can be shown and/or given to each mother to help her remember appropriate food and fluids, and when to return to the health worker. An example Mother's Card is shown in **Annex C**. The Mother's Card has words and pictures that illustrate the main points of advice.

There are many reasons a Mother's Card can be helpful:

- It will remind you of important points to cover when counselling mothers about foods, fluid, and when to return.
- It will remind the mother what to do when she gets home.
- The mother may show the card to other family members or neighbours, so more people will learn the messages it contains.
- The mother will appreciate being given something during the visit.
- Multivisit cards can be used as a record of treatments and immunizations given.

When reviewing a Mother's Card with a mother:

1. Hold the card so the mother can easily see the pictures, or allow her to hold it herself.
2. Explain each picture. Point to the pictures as you talk. This will help the mother remember what the pictures represent.
3. Circle or record information that is relevant to the mother. For example, circle the feeding advice for the child's age. Circle the signs to return immediately. If the child has diarrhoea, tick the appropriate fluid(s) to give. Record the date of the next immunization needed.
4. Watch to see if the mother seems worried or puzzled. If so, encourage questions.
5. Ask the mother to tell you in her own words what she should do at home. Encourage her to use the card to help her remember.
6. Give her the card to take home. Suggest that she show it to others in her family.

If you cannot obtain a large enough supply of cards to give to every mother, keep several in the clinic to show to mothers.

29.6 Advise the mother to increase fluid during illness

During illness an infant or young child loses fluid due to fever, fast breathing, or diarrhoea. The child will feel better and stay stronger if he or she drinks extra fluid to prevent dehydration. Frequent breastfeeding will give the infant nourishment and help prevent dehydration.

Extra fluid is especially important for children with diarrhoea; these children should be given fluid according to Plan A or B as described on the *TREAT* chart. Mothers of breastfeeding children should offer the breast frequently.

Advice about fluid is summarized in the *COUNSEL THE MOTHER* chart and in the *TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER* section of the *YOUNG INFANT* chart. Give this advice to every mother who is taking her child home UNLESS she has already received many instructions and may be overwhelmed by more advice, or has already been taught Plan A.

CHAPTER 30

Counsel the mother about when to return and about her own health

Every mother or caretaker who is taking a sick young infant or child home needs to be advised about when to return to a health facility. You should advise her when to return for follow-up visits and teach her signs that mean to return immediately for further care.

The mother or caretaker may need to return:

1. for a **FOLLOW-UP VISIT** in a specific number of days (for example, when it is necessary to check progress on an antibiotic),
2. **IMMEDIATELY**, if signs appear that suggest the illness is worsening, or
3. for the child's **NEXT IMMUNIZATION** (the next **WELL-CHILD VISIT**).

30.1 Advise when to return for a follow-up visit

Certain problems require follow-up in a specific number of days. For example, pneumonia, dysentery and acute ear infection require follow-up to ensure that an antibiotic is working. Persistent diarrhoea requires follow-up to ensure that feeding changes are working. Some other problems, such as fever or pus draining from the eye, require follow-up only if the problem persists.

At the end of the visit, tell the mother or caretaker when to return for follow-up. Sometimes an infant or child may need follow-up for more than one problem. In such cases, tell the mother the **earliest definite** time to return. Also tell her about any earlier follow-up that may be needed if a problem, such as fever, persists.

Both the *COUNSEL THE MOTHER* chart and the *YOUNG INFANT* chart show summaries of the follow-up times for different problems.

Notice that there are several different follow-up times related to nutrition:

- If a child has a feeding problem and you have recommended changes in feeding, follow-up in 5 days to see if the mother has made the changes. You will give more counselling if needed.
- If a child has pallor, follow-up in 14 days to give more iron.
- If the child has **VERY LOW WEIGHT**, additional follow-up is needed in 30 days. This follow-up would involve weighing the child, re-assessing feeding practices, and giving any further advice needed from the *COUNSEL* chart.

If a clinic has a regular session reserved for counselling about feeding, schedule follow-up visits for that time. If such sessions are not offered, schedule an individual visit for feeding counselling at a time when a health worker will be available to discuss feeding with the mother. This health worker will need to know about the child's feeding problems, changes recommended, and the child's weight. This information can be recorded in the patient chart, or in a special follow-up note.

30.2 Advise when to return immediately

For all infants and children who are going home, you will advise the mother when to return immediately. This means to teach the mother or caretaker certain signs that mean to return immediately for further care. These signs are listed in the section **WHEN TO RETURN** on both the *COUNSEL THE MOTHER* and *YOUNG INFANT* charts. Remember that this is an extremely important section.

For sick **young infants** age 1 week up to 2 months, refer to the *YOUNG INFANT* chart and teach the mother the following signs to watch for:

WHEN TO RETURN IMMEDIATELY

Advise the mother to return immediately if the young infant has any of these signs:
Breastfeeding or drinking poorly Becomes sicker Develops a fever Fast breathing Difficult breathing Blood in stool

In Addition: advise the mother to make sure the infant stays warm at all times. Keeping a sick young infant warm (but not too warm) is very important. Low temperature alone can kill young infants.

For sick **children** age 2 months up to 5 years, refer to the *COUNSEL THE MOTHER* chart and teach the mother the appropriate signs to watch for:

WHEN TO RETURN IMMEDIATELY

Advise mother to return immediately if the child has any of these signs:	
Any sick child	● Not able to drink or breastfeed ● Becomes sicker ● Develops a fever
If child has NO PNEUMONIA: COUGH OR COLD, also return if:	● Fast breathing ● Difficult breathing
If child has Diarrhoea, also return if:	● Blood in stool ● Drinking poorly

Exceptions: If the child already has fever, you do not need to tell the mother to return immediately for fever. If the child already has blood in the stool, you do not need to tell the mother to return immediately for blood, just for drinking poorly.

The signs mentioned above are particularly important signs to watch for. Use the Mother's Card to explain the signs and to help the mother or caretaker remember them (see **Annex C**). The Mother's Card presents signs in both words and drawings. Circle the signs that the mother must remember. Use local terms that she will understand. Ask her checking questions to be sure that she understands.

30.3 Advise when to return for the next well-child visit

Remind the mother or caretaker of the next visit her child needs for immunization **unless** the mother already has a lot to remember and will return soon anyway. For example, if a mother must remember a schedule for giving an antibiotic, home care instructions for another problem, and a follow-up visit in 2 days, do not describe a well-child visit needed one month from now. However, do **record the date of the next immunization** on the Mother's Card.

30.4 Counsel the mother about her own health

During a sick child visit, listen for any problems that the mother herself may be having. The mother may need treatment or referral for her own health problems.

Part VII

GIVE FOLLOW-UP CARE

CHAPTER 31

Follow-up care for the sick child

Some sick children need to return to the health worker for follow-up. Their mothers are told when to come for a follow-up visit (such as in 2 days, or 14 days). At a follow-up visit the health worker can see if the child is improving on the drug or other treatment that was prescribed. Some children may not respond to a particular antibiotic or anti-malarial and may need to try a second drug. Children with persistent diarrhoea also need follow-up to be sure that the diarrhoea has stopped. Children with fever or eye infection need to be seen if they are not improving. Follow-up is especially important for children with a feeding problem; to be sure they are being fed adequately and are gaining weight.

Because follow-up is important, you should make special arrangements so that follow-up visits are convenient for mothers. If possible, mothers should not have to wait in the queue for a follow-up visit. Not charging for follow-up visits is another way to make follow-up convenient and acceptable for mothers. Some clinics use a system that makes it easy to find the records of children scheduled for follow-up.

At a follow-up visit, you should do different steps than at a child's initial visit for a problem. Treatments given at the follow-up visit are often different than those given at an initial visit.

Where is follow-up discussed on the case management charts?

In the “Identify Treatment” column of the *ASSESS & CLASSIFY* chart, some classifications have instructions to tell the mother to return for follow-up. The “When to Return” box on the *COUNSEL* chart summarizes the schedules for follow-up visits.

FOLLOW-UP VISIT TABLE IN THE COUNSEL THE MOTHER CHART

FOLLOW-UP VISIT

Advise the mother to come for follow-up at the earliest time listed for the child's problems:

If the child has:	Return for follow-up in:
PNEUMONIA DYSENTERY MALARIA, if fever persists FEVER—MALARIA UNLIKELY, if fever persists MEASLES WITH EYE OR MOUTH COMPLICATIONS	2 days
PERSISTENT DIARRHOEA ACUTE EAR INFECTION CHRONIC EAR INFECTION FEEDING PROBLEM ANY OTHER ILLNESS, if not improving	5 days
PALOR	14 days
VERY LOW WEIGHT FOR AGE	30 days

Specific instructions for conducting each follow-up visit are in the “Give Follow-Up Care” section of the *TREAT THE CHILD* chart. The boxes have headings that correspond to the classifications on the *ASSESS & CLASSIFY* chart. Each box tells how to reassess and treat the child. Instructions for giving treatments, such as drug dosages for a second-line antibiotic or antimalarial, are on the *TREAT THE CHILD* chart.

Follow-up instructions for young infants are on the *YOUNG INFANT* chart and are described in **Chapter 32**.

31.1 How to manage a child who comes for follow-up

As always, ask the mother about the child’s problem. You need to know if this is a follow-up or an initial visit for this illness. How you find out depends on how your clinic registers patients and how the clinic finds out why they have come.

For example, the mother may say to you or other clinic staff that she was told to return for follow-up for a specific problem. If your clinic gives mothers follow-up slips that tell them when to return, ask to see the slip. If your clinic keeps a chart on each patient, you may see that the child came only a few days ago for the same illness.

Once you know that the child has come to the clinic for follow-up of an illness, ask the mother if the child has, in addition, developed any **new** problems. For example, if the child has come for follow-up of pneumonia, but now he has developed diarrhoea, he has a **new** problem. This child requires a full assessment. Check for general danger signs and assess all the main symptoms and the child’s nutritional status. Classify and treat the child for diarrhoea (the new problem) as you would at an initial visit. Reassess and treat the pneumonia according to the follow-up box.

If the child does *not* have a new problem, locate the follow-up box that matches the child’s previous classification. Then follow the instructions in that box.

- Assess the child according to the instructions in the follow-up box. The instructions may tell you to assess a major symptom as on the *ASSESS & CLASSIFY* chart. They may also tell you to assess additional signs.

Note: Do not use the classification table to classify a main symptom. Skip the “Classify” and “Identify Treatment” columns on the *ASSESS & CLASSIFY* chart. This will avoid giving the child repeated treatments that do not make sense. There is one exception: If the child has any kind of diarrhoea, classify and treat the dehydration as you would at an initial assessment.

- Use the information about the child’s signs to select the appropriate treatment.
- Give the treatment.

Some children will return repeatedly with chronic problems that do not respond to the treatment that you can give. For example, some children with AIDS may have persistent diarrhoea or repeated episodes of pneumonia. Children with AIDS may respond poorly to treatment for pneumonia and may have opportunistic infections. These children should be referred to hospital when they do not improve. Children with HIV infection who have not developed AIDS cannot be clinically distinguished from those without HIV infection. When they develop pneumonia, they respond well to standard treatment.

Important: If a child who comes for follow-up has several problems and is getting worse, REFER THE CHILD TO HOSPITAL. Also refer the child to hospital if a second-line drug is not available, or if you are worried about the child or do not know what to do for the child. If a child has not improved with treatment, the child may have a different illness than suggested by the chart. He may need other treatment.

Remember:
If a child has any new problem, you should assess that child as at an initial visit

31.2 Conduct a follow-up visit for pneumonia

When a child receiving an antibiotic for PNEUMONIA returns to the clinic after 2 days for follow-up, follow these instructions:

The box first describes how to assess the child. It says to check the child for general danger signs and reassess the child for cough and difficult breathing. Next to these instructions, it says to see the *ASSESS & CLASSIFY* chart. This means that you should assess general danger signs and the main symptom cough exactly as described on the *ASSESS & CLASSIFY* chart. Then it lists some additional items to check:

Ask:

- Is the child breathing slower?
- Is there less fever?
- Is the child eating better?

When you have assessed the child, use the information about the child's signs to select the correct treatment.

- ▶ If the child has **chest indrawing or a general danger sign** (not able to drink or breastfeed, vomits everything, convulsions, lethargic or unconscious) the child is getting worse. This child needs URGENT referral to a hospital. Since the illness has worsened on the first-line antibiotic for pneumonia, give the first dose of the second-line antibiotic (if available) or give intramuscular chloramphenicol before referral.
- ▶ If **breathing rate, fever, and eating are the same**, give the child the second-line antibiotic for pneumonia. (The signs may not be *exactly* the same as 2 days before, but the child is not worse and not improving. The child still has fast breathing, fever and poor eating.) However, before you give the second-line antibiotic, ask the mother if the child took the antibiotic for the previous 2 days.
 - a) There may have been a problem so that the child did not receive the antibiotic, or received too low or too infrequent a dose. If so, this child can be treated again with the same antibiotic. Give a dose in clinic, and check that the mother knows how to give the drug at home. Help her to solve any problems such as how to encourage the child to take the drug when the child refuses it.
 - b) If the child received the antibiotic, change to the second-line antibiotic for pneumonia, if available in your clinic. Give it for 5 days. For example:
 - If the child was taking cotrimoxazole, switch to amoxycillin.
 - If the child was taking amoxycillin, switch to cotrimoxazole.
 Give the first dose of the antibiotic in the clinic. Teach the mother how and when to give it. Ask the mother to bring the child back again in 2 more days.
 - c) If the child received the antibiotic, and you do not have another appropriate antibiotic available, refer the child to a hospital.

If a child with pneumonia had measles within the last 3 months, refer the child to hospital.

- ▶ If the child is **breathing slower, has less fever** (that is, the fever is lower or is completely gone) and is **eating better**, the child is **improving**. The child may cough, but most children who are improving will no longer have fast breathing. Tell the mother that the child should finish taking the 5 days of the antibiotic. Review with her the importance of finishing the entire 5 days.

31.3 Conduct a follow-up visit for persistent diarrhoea

Refer to the “Persistent Diarrhoea” box in the follow-up section of the *TREAT THE CHILD* chart. When a child with PERSISTENT DIARRHOEA returns for a follow-up visit after 5 days, follow the instructions below.

Ask if the diarrhoea has stopped and how many stools the child has per day.

- If the diarrhoea has not stopped (the child is still having 3 or more loose stools per day), do a full reassessment. This should include assessing the child completely as described on the *ASSESS & CLASSIFY* chart. Identify and manage any problems that require immediate attention such as dehydration. Then refer the child to hospital.
- If the diarrhoea has stopped (child having less than 3 loose stools per day), instruct the mother to follow the feeding recommendations for the child's age. If the child is not normally fed in this way, you will need to teach her the feeding recommendations on the *COUNSEL* chart.

31.4 Conduct a follow-up visit for dysentery

Refer to the "Dysentery" box in the follow-up section of the *TREAT THE CHILD* chart. When a child classified as having DYSENTERY returns for a follow-up visit after 2 days, follow the instructions below.

Reassess the child for diarrhoea as described in the box, "Does the child have diarrhoea?" on the *ASSESS & CLASSIFY* chart. Ask the mother the additional questions to find out if the child is improving. Then use the information about the child's signs to decide if the child is the same, worse, or better. Select the appropriate treatment:

- If the child is **dehydrated** at the follow-up visit, use the classification table to classify the child's dehydration. Select the appropriate fluid plan and treat the dehydration.
- If the **number of stools, amount of blood in stools, fever, abdominal pain, or eating is the same or worse**, stop the first antibiotic and give the second-line antibiotic recommended for *Shigella*. (This antibiotic will be specified on the *TREAT* chart.) Antibiotic resistance of *Shigella* may cause the lack of improvement.

— Give the first dose of the new antibiotic in the clinic.

— Teach the mother how and when to give the antibiotic and help her plan how to give it for 5 days.

— Advise the mother to bring the child back again after two more days.

If after being treated with the second-line antibiotic for two days the child has still not improved, the child may have amoebiasis. This child may be treated with metronidazole (if it is available or can be obtained by the family) or referred for treatment. Amoebiasis can only be diagnosed with certainty when trophozoites of *E. histolytica* containing red blood cells are seen in a fresh stool sample.

However, if the child is

- less than 12 months old, or
- was dehydrated on the first visit, or
- had measles within the last 3 months,

this child is at high risk. Refer this child to hospital.

- If the child has **fewer stools, less blood in the stools, less fever, less abdominal pain, and is eating better**, the child is improving on the antibiotic. Usually all of these signs will diminish if the antibiotic is working. If only some signs have diminished, use your judgement to decide if the child is improving. Tell the mother to finish the 5 days of the antibiotic. Review with the mother the importance of finishing the antibiotic.

31.5 Conduct a follow-up visit for malaria (low or high malaria risk)

Any child classified as having **MALARIA** (regardless of the risk of malaria) should return for a follow-up visit if the fever persists for 2 days. If the fever persists 2 days after the initial visit or if the fever returns within 14 days, this may mean that the child has a

malaria parasite which is resistant to the first-line antimalarial, causing the child's fever to continue.

If the child also had MEASLES at the initial visit, the fever may be due to measles. It is very common for the fever from measles to continue for several days. Therefore, the persistent fever may be due to the measles rather than to resistant malaria.

The instructions for conducting a follow-up visit for a child classified as having MALARIA are the same for low or high malaria risk. Refer to the "Malaria" box in the follow-up section of the *TREAT THE CHILD* chart.

Do a full reassessment of the child as on the *ASSESS & CLASSIFY* chart. As you reassess the child, look for the cause of the fever, possibly pneumonia, meningitis, measles, ear infection, or dysentery. Also consider whether the child has any other problem that could cause the fever, such as tuberculosis, urinary tract infection, osteomyelitis or abscess. Do not use the classification table of the *ASSESS & CLASSIFY* chart to classify the child's fever. Instead, choose the appropriate treatment shown in the follow-up box. If you suspect a cause of fever other than malaria, assess the problem further if needed and refer to any guidelines on treatment of the problem.

- If the child has **any general danger signs or stiff neck**, treat as described on the *ASSESS & CLASSIFY* chart for VERY SEVERE FEBRILE DISEASE. This includes giving quinine, a first dose of an antibiotic and a dose of paracetamol. Also treat to prevent low blood sugar and refer urgently to hospital. If the child has already been on an antibiotic, worsening of the illness to very severe febrile disease means he may have a bacterial infection that is not responsive to this anti-biotic. Give a first dose of the second-line antibiotic or intramuscular chloramphenicol. If the child cannot take an oral antibiotic because he has repeated vomiting, is lethargic or unconscious, or is not able to drink, give intramuscular chloramphenicol. Also give intramuscular chloramphenicol if he has a stiff neck.
- If the child has **any cause of fever other than malaria**, provide treatment for that cause. For example, give treatment for the ear infection or refer for other problems such as urinary tract infection or abscess.
- If **malaria is the only apparent cause of fever**:
 - Treat with second-line oral antimalarial. If this is not available, refer the child to hospital. Ask the mother to return again in 2 days if the fever persists.
 - If the fever has been present every day for 7 days or more, refer the child for assessment. This child may have typhoid fever or another serious infection requiring additional diagnostic testing and special treatment.

***Note:** If the child has been taking cotrimoxazole because he also had cough and fast breathing (pneumonia) as well as fever, give the second-line antimalarial unless it is sulfadoxine-pyrimethamine. Because cotrimoxazole (trimethoprim-sulfamethoxazole) and sulfadoxine-pyrimethamine are closely related drugs, the two should not be taken together. If malaria has not improved on cotrimoxazole, sulfadoxine-pyrimethamine will not be effective either. Refer this child to hospital.*

Continue giving cotrimoxazole to this child if the child's pneumonia is improving. Otherwise, the second-line antibiotic may be needed as well, as described in section 31.2.

31.6 Conduct a follow-up visit for fever—malaria unlikely (low or no malaria risk)

Refer to the "Fever—Malaria Unlikely" box in the follow-up section of the *TREAT THE CHILD* chart. When a child whose fever was classified as FEVER—MALARIA UNLIKELY returns for follow-up after 2 days because the fever persists, follow the instructions below.

When a child come from a low or no malaria risk area, and fever persists after 2 days, there may be some cause of fever that was not apparent at the first visit. Do a full reassessment of the child as on the *ASSESS & CLASSIFY* chart. Look for the cause of fever. Also consider whether the child has any other problem that caused the fever, such as tuberculosis, urinary tract infection, osteomyelitis or abscess. Then select the appropriate treatment in the follow-up boxes.

- If the child has **any general danger sign or stiff neck**, treat as VERY SEVERE FEBRILE DISEASE.
- If the child has **any cause of fever other than malaria**, provide treatment or refer for care of that cause.
- If **malaria is the only apparent cause of fever**, treat the child with the first-line oral antimalarial recommended by national policy to cover the possibility of malaria. Advise the mother to return again in 2 days if the fever persists.

If the fever has been present every day for 7 days, refer the child. Further diagnostic tests are needed to determine the cause of this child's persistent fever.

31.7 Conduct a follow-up visit for measles with eye or mouth complications

Refer to the “Measles with Eye or Mouth Complications” box in the follow-up section of the *TREAT THE CHILD* chart. When a child who was classified as having MEASLES WITH EYE OR MOUTH COMPLICATIONS returns for follow-up in 2 days, follow the instructions below.

To assess the child, check the eyes and mouth. Select treatment based on the child's signs.

Treatment for eye infection:

- If **pus is still draining from the eye**, ask the mother to describe or show you how she has been treating the eye infection. If she has brought the tube of ointment with her, you can see whether it has been used. There may have been problems so that the mother did not do the treatment correctly. For example, she may not have treated the eye three times a day, or she may not have cleaned the eye before applying the ointment, or the child may have struggled so that she could not put the ointment in the eye.
 - If the mother has correctly treated the eye infection for 2 days and there is still pus draining from the eye, refer the child to a hospital.
 - If the mother has not correctly treated the eye, ask her what problems she had in trying to give the treatment. Teach her any parts of the treatment that she does not seem to know. Discuss with her how to overcome difficulties she is having. Finally, explain to her the importance of the treatment. Ask her to return again if the eye does not improve. However, if you think that the mother still will not be able to treat the eye correctly, arrange to treat the eye each day in clinic or refer the child to a hospital.
- If **pus is gone but redness remains**, continue the treatment. Tell the mother that the treatments are helping. Encourage her to continue giving the correct treatment until the redness is gone.
- If **no pus or redness**, stop the treatment. Praise the mother for treating the eye well. Tell her the infection is gone.

Treatment for mouth ulcers:

- If **mouth ulcers are worse, or there is a very foul smell from the mouth**, refer to hospital. The mouth problem may prevent the child from eating or drinking and

may become severe. A very foul smell may mean a serious infection. Mouth problems of measles could be complicated by thrush or herpes (the virus which causes cold sores).

- If **mouth ulcers are the same or better**, ask the mother to continue treating the mouth with half-strength gentian violet for a total of 5 days.

She should continue to feed the child appropriately to make up for weight lost during the acute illness and to prevent malnutrition. Review with the mother when to seek care and how to feed her child as described on the *COUNSEL THE MOTHER* chart. Tell her that attention to feeding is especially important for children who have measles because they are at risk of developing malnutrition.

Because the child with measles continues to have increased risk of illness for months, it is important that the mother know the signs to bring the child back for care. Children who have measles are at increased risk of developing complications or a new problem, due to immune suppression that occurs during and following measles.

31.8 Conduct a follow-up visit for ear infection

Refer to the “Ear Infection” box in the follow-up section of the *TREAT THE CHILD* chart. When a child classified as EAR INFECTION returns for a follow-up visit after 5 days, follow the instructions below. These instructions apply to an acute or a chronic ear infection.

Reassess the child for ear problem and measure the child’s temperature (or feel the child for fever). Then select treatment based on the child’s signs.

- If you feel a **tender swelling behind the ear** when compared to the other side, the child may have developed mastoiditis. If there is a **high fever** (an axillary temperature of 38.5 °C or above), the child may have a serious infection. A child with tender swelling behind the ear or high fever has gotten worse, and should be referred to a hospital.
- **Acute ear infection:** If **ear pain or ear discharge persists** after taking an antibiotic for 5 days, treat with 5 additional days of the same antibiotic. Ask the mother to return in 5 more days so that you can check whether the ear infection is improving.
If the ear is still draining or has begun draining since the initial visit, show the mother how to wick the ear dry. Discuss with her the importance of keeping the ear dry so that it will heal.
- **Chronic ear infection:** Check that the mother is wicking the ear correctly. To do this, ask her to describe or show you how she wicks the ear. Ask her how frequently she is able to wick the ear. Ask her what problems she has in trying to wick the ear and discuss with her how to overcome them. Encourage her to continue wicking the ear. Explain that drying is the only effective therapy for a draining ear. Not wicking the ear could leave the child with reduced hearing.
- If **no ear pain or discharge**, praise the mother for her careful treatment. Ask her if she has given the child the 5 days of antibiotic. If not, tell her to use all of it before stopping.

31.9 Conduct a follow-up visit for feeding problem

Refer to the “Feeding Problem” box in the follow-up section of the *TREAT THE CHILD* chart. When a child who had a feeding problem returns for follow-up in 5 days, follow the instructions below.

Reassess the child’s feeding by asking the questions in the top box on the *COUNSEL THE MOTHER* chart. Refer to the child’s chart or follow-up note for a description of

any feeding problems found at the initial visit and previous recommendations. Ask the mother how she has been carrying out the recommendations. For example, if on the last visit more active feeding was recommended, ask the mother to describe how and by whom the child is fed at each meal.

- Counsel the mother about any new or continuing feeding problems. If she encountered problems when trying to feed the child, discuss ways to solve them.

For example, if the mother is having difficulty changing to more active feeding because it requires more time with the child, discuss some ways to reorganize the meal time.

- If the child is very low weight for age, ask the mother to return 30 days after the initial visit. At that visit a health worker will measure the child's weight gain to determine if the changes in feeding are helping the child.

Example:

On the initial visit the mother of a 2-month-old infant said that she was giving the infant 2 or 3 bottles of milk and breastfeeding several times each day. The health worker advised the mother to give more frequent, longer breastfeeds and gradually reduce other milk or foods.

At the follow-up visit, the health worker asks the mother questions to find out how often she is giving the other feeds and how often and for how long she is breastfeeding. The mother says that she now gives the infant only 1 bottle of milk each day and breastfeeds 6 or more times in 24 hours. The health worker tells the mother that she is doing well. The health worker then asks the mother to completely stop the other milk and breastfeed 8 or more times in 24 hours. Since this is a significant change in feeding, the health worker also asks the mother to come back again. At that visit the health worker will check that the infant is feeding frequently enough and encourage the mother.

31.10 Conduct a follow-up visit for pallor

Refer to the "Pallor" box in the follow-up section of the *TREAT THE CHILD* chart. When a child who had palmar pallor returns for a follow-up visit after 14 days, follow the instructions below.

- Give the mother additional iron for the child and advise her to return in 14 days for more iron.
- Continue to give the mother iron when she returns every 14 days for up to 2 months.
- If after 2 months the child still has palmar pallor, refer the child for assessment.

31.11 Conduct a follow-up visit for very low weight

A child who was classified with VERY LOW WEIGHT should return for follow-up after 30 days (the child would also return earlier if there was a feeding problem). Some clinics have specially scheduled sessions for nutritional counselling, and malnourished children are asked to come for follow-up at this time. A special session allows the health worker to devote the necessary time to discuss feeding with several mothers and perhaps demonstrate some good foods for young children.

Refer to the "Very Low Weight" box in the follow-up section of the *TREAT THE CHILD* chart. Follow the instructions below for a follow-up visit for a child with VERY LOW WEIGHT.

To assess the child, weigh him and determine if the child is still very low weight for age. Also reassess feeding by asking the questions in the top box of the *COUNSEL* chart.

- ▶ If the child is **no longer very low weight for age**, praise the mother. The changes in the child's feeding are helping. Encourage her to continue feeding the child according to the recommendation for his age.
- ▶ If the child is still **very low weight for age**, counsel the mother about any feeding problem found. This nutritional counselling should include teaching the mother to feed the child the foods appropriate for his age and to give them frequently enough. It should also include teaching her how to feed him actively.
- ▶ Ask the mother to bring the child back again in one month. It is important to continue seeing the child every month to advise and encourage the mother until he is feeding well and gaining weight regularly or is no longer very low weight. If the child is continuing to lose weight and no change in feeding seems likely, refer the child to hospital or to a feeding programme.

CHAPTER 32

Follow-up care for the sick young infant

Follow-up visits are recommended for young infants who are classified as **LOCAL BACTERIAL INFECTION**, **DYSENTERY**, **FEEDING PROBLEM OR LOW WEIGHT** (including thrush). Instructions for carrying out follow-up visits for the sick young infant age 1 week up to 2 months are on the *YOUNG INFANT* chart.

As with the sick child who comes for follow-up, a sick young infant is assessed differently at a follow-up visit than at an initial visit. Once you know that the young infant has been brought to the clinic for follow-up, ask whether there are any **new** problems. An infant who has a new problem should receive a full assessment as if it were an initial visit.

If the infant does not have a new problem, locate the section of the *YOUNG INFANT* chart with the heading “Give Follow-Up Care for the Sick Young Infant.” Use the box that matches the infant’s previous classification.

The instructions in the follow-up box (for the previous classification) tell how to assess the young infant. These instructions also tell the appropriate follow-up treatment to give. Do not use the classification tables for the young infant to classify the signs or determine treatment. There is one exception: If the young infant has dysentery, classify and treat dehydration as you would at an initial assessment.

32.1 Dysentery

When a young infant classified as having **DYSENTERY** returns for follow-up in 2 days, follow the instructions in the “Dysentery” box on the follow-up section of the chart.

Reassess the young infant for diarrhoea as described in the assessment box, “Does the young infant have diarrhoea?” Also, ask the mother the additional questions listed to determine whether the infant is improving.

- If the infant is **dehydrated**, use the classification table on the *YOUNG INFANT* chart to classify the dehydration and select a fluid plan.
- If the signs are the same or worse, refer the infant to hospital. If the young infant has developed fever, give intramuscular antibiotics before referral, as for **POSSIBLE SERIOUS BACTERIAL INFECTION**.
- If the infant’s signs are improving, tell the mother to continue giving the infant the antibiotic. Make sure the mother understands the importance of completing the 5 days of treatment.

32.2 Local bacterial infection

When a young infant classified as having **LOCAL BACTERIAL INFECTION** returns for follow-up in 2 days, follow the instructions in the “Local Bacterial Infection” box of the follow-up section of the chart.

To assess the young infant, look at the umbilicus or skin pustules. Then select the appropriate treatment.

- If **pus or redness remains or is worse**, refer the infant to hospital. Also refer if there are more pustules than before.

- If **pus and redness are improved**, tell the mother to complete the 5 days of antibiotic that she was given during the initial visit. Improved means there is less pus and it has dried. There is also less redness. Emphasize that it is important to continue giving the antibiotic even when the infant is improving. She should also continue treating the local infection at home for 5 days (cleaning and applying gentian violet to the skin pustules or umbilicus).

32.3 Feeding problem

When a young infant who had a feeding problem returns for follow-up in 2 days, follow the instructions in the “Feeding Problem” box on the follow-up section of the chart. Reassess the feeding by asking the questions in the young infant assessment box, “Then Check for Feeding Problem or Low Weight.” Assess breastfeeding if the infant is breastfed.

Refer to the young infant’s chart or follow-up note for a description of the feeding problem found at the initial visit and previous recommendations. Ask the mother how successful she has been carrying out these recommendations and ask about any problems she encountered in doing so.

- Counsel the mother about new or continuing feeding problems. Refer to the recommendations in the box “Counsel the Mother About Feeding Problems” on the *COUNSEL* chart and the box “Teach Correct Positioning and Attachment for Breastfeeding” on the *YOUNG INFANT* chart.

For example, you may have asked a mother to stop giving an infant drinks of water or juice in a bottle, and to breastfeed more frequently and for longer. You will assess how many times she is now breastfeeding in 24 hours and whether she has stopped giving the bottle. Then advise and encourage her as needed.

- If the young infant is low weight for age, ask the mother to return 14 days after the initial visit. At that time, you will assess the young infant’s weight again. Young infants are asked to return sooner to have their weight checked than older infants and young children are. This is because they should grow faster and are at higher risk if they do not gain weight.

32.4 Low weight

When a young infant who was classified as LOW WEIGHT returns for follow-up in 14 days, follow the instructions in the “Low Weight” box on the follow-up section of the chart.

Determine if the young infant is still low weight for age. Also reassess his feeding by asking the questions in the assessment box, “Then Check for Feeding Problem or Low Weight.” Assess breastfeeding if the young infant is breastfed.

- If the young infant is **no longer low weight for age**, praise the mother for feeding the infant well. Encourage her to continue feeding the infant as she has been or with any additional improvements you have suggested.
- If the young infant is **still low weight for age, but is feeding well**, praise the mother. Ask her to have her infant weighed again within a month or when she returns for immunization. You will want to check that the infant continues to feed well and continues gaining weight. Many young infants who were low birthweight will still be low weight for age, but will be feeding and gaining weight well.
- If the young infant is **still low weight for age and still has a feeding problem**, counsel the mother about the problem. Ask the mother to return with her infant again in 14 days. Continue to see the young infant every few weeks until you are sure he is feeding well and gaining weight regularly or is no longer low weight for age.

32.5 Thrush

When a young infant who had thrush returns for follow-up in 2 days, follow the instructions in the “Thrush” box on the follow-up section of the chart.

Check the thrush and reassess the infant’s feeding.

- If the **thrush is worse** or the infant has **problems with attachment or suckling**, refer to hospital. It is very important that the infant be treated so that he can resume good feeding as soon as possible.
- If the **thrush is the same or better** and the infant is **feeding well**, continue the treatment with half-strength gentian violet. Stop using gentian violet after 5 days.

ANNEX A: **PLAN C – TREAT SEVERE DEHYDRATION QUICKLY**

1. If you can give intravenous (IV) treatment

If you can give IV treatment and you have acceptable solutions such as Ringer's Lactate or Normal Saline at your clinic, give the solution intravenously to the severely dehydrated child.¹

The sections of Plan C below describe the steps to rehydrate a child intravenously. It includes the amounts of IV fluid that should be given according to the age and weight of the child. Study the sections carefully.

- Start IV fluid immediately. If the child can drink, give ORS by mouth while the drip is set up. Give 100ml/kg Ringer's Lactate Solution (or, if not available, normal saline), divided as follows:

AGE	First give 30 ml/kg in:	Then give 70 ml/kg in:
Infants (under 12 months)	1 hour*	5 hours
Children (12 months up to 5 years)	30 minutes*	2 1/2 hours

** Repeat once if radial pulse is still very weak or not detectable.*

- Reassess the child every 1–2 hours. If hydration status is not improving, give the IV drip more rapidly.
- Also give ORS (about 5 ml/kg/hour) as soon as the child can drink: usually after 3–4 hours (infants) or 1–2 hours (children).
- Reassess an infant after 6 hours and a child after 3 hours. Classify dehydration. Then choose the appropriate plan (A, B, or C) to continue treatment.

Note:

- If possible, observe the child at least 6 hours after rehydration to be sure the mother can maintain hydration giving the child ORS solution by mouth.

Some of the terms in this part of Plan C may be new to you. Read the following to understand how the terms are used in Plan C.

- The DRIP refers to the IV equipment and solution.
- The “rate of the drip” refers to the number of drops per minute that the IV fluid is given.
- “While the drip is set up” means during the time you are preparing the IV equipment, IV fluid and you are putting the IV needle into the child's vein.

¹ This annex will not teach how to give intravenous treatment.

- **HYDRATION STATUS** refers to whether the child is normally hydrated or dehydrated and the extent of dehydration. A child classified as **NO DEHYDRATION** has not lost enough fluid to show signs of dehydration. A child classified as **SOME DEHYDRATION** or **SEVERE DEHYDRATION** has less than a normal amount of fluid in the body.
- To assess a child's hydration status, refer to the signs on the *ASSESS & CLASSIFY* chart.
- The **RADIAL PULSE** refers to the pulse felt over the radial artery. The radial artery is the main blood vessel at the wrist on the side of the thumb.

Provide IV treatment for severe dehydration

When you provide IV therapy for **SEVERE DEHYDRATION**, you give the child a large quantity of fluids quickly. The fluids replace the body's very large fluid loss.

Begin IV treatment quickly in the amount specified in Plan C. If the child can drink, give ORS by mouth until the drip is running. Then give the first portion of the IV fluid (30 ml/kg) very rapidly (within 60 minutes for infants, within 30 minutes for children). This will restore the blood volume and prevent death from shock. Then give 70 ml/kg more slowly to complete rehydration.

During the IV treatment, assess the child every 1–2 hours. Determine if the child is receiving an adequate amount of IV fluid.

Monitor amount of IV fluid and the child's hydration status

When rehydrating a child who has **SEVERE DEHYDRATION**, you have to monitor the amount of IV fluid that you give. You may use a form, similar to the following sample form.

Time (hr)	Volume (ml) Set-up*	Estimated Volume (ml) Remaining	Volume (ml) Received
_____	_____		
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

* For each new bottle/pack, initial or added

The form has 4 columns to record the amount of fluids given to a patient over a period of time.

1. **Time:** Record the times that you will check the IV fluid.

For an Infant: (under 12 months)

* After the first hour

* After the first half hour (30 minutes)

For a Child: (12 months up to 5 years)

* After the first half hour (30 minutes)

* Every hour over the next 5 hours

2. **Volume Set-up:** As you start the IV fluid, record the amount of fluid in the bottle or pack. The amount should be listed on the container. Each time you replace the IV fluid with another container, be sure to record the amount on the appropriate line on the form at the time of replacement.
3. **Estimated Volume Remaining:** Check the NG fluid remaining in the container at the times listed. The remaining volume cannot be read precisely. Estimate it to the nearest 10 ml (for example—220 ml, 230 ml, 240 ml, etc). Record the estimated amount on the form.
4. **Volume Received:** Calculate the amount of IV fluid received by the child at the times listed. To calculate, subtract the “Volume remaining” amount from the “Volume set-up” amount. The answer is the amount of IV fluid the child has received up to the time you are checking. Record that amount on the form.

It is helpful to mark the IV fluid container with a pen or tape to show the level that should be reached at a certain time. For example, mark the desired level to reach after the first 30 or 60 minutes, each hour, or at the end of 3 or 6 hours. This will help you adjust the rate of the drip correctly. Regulate the number of drops per minute to give the correct amount of fluid per hour.

The sample form below shows the amounts of IV fluid given to a 16-month-old (10 kg) child who is classified as having SEVERE DEHYDRATION. The health worker followed Plan C. He gave the child 300 ml (30 ml x 10 kg) in the first 30 minutes. He gave 700 ml (70 ml x 10 kg) over the next 2.5 hours (about 300 ml per hour).

SAMPLE FLUID FORM

Time (hr)	Volume (ml) Set-up*	Estimated Volume (ml) Remaining	Volume (ml) Received
12:00 pm	1000 ml		
12:30 pm		700 ml	300 ml
1:30 pm		400 ml	600 ml
2:30 pm		100 ml	900 ml
3:00 pm		0 ml	1000 ml

* For each new bottle/pack, initial or added

Make sure the IV fluid is given correctly and in adequate amounts. To monitor whether the fluid rate is adequate, reassess the child's dehydration every 1–2 hours. If the signs of dehydration and the diarrhoea are worse or not improved, increase both the rate you give the fluid and the amount of fluid that you give. Also increase the fluid rate if the child is vomiting. If the signs are improving, continue giving IV fluid at the same rate.

While giving IV fluid, remember to also give small sips of ORS solution to the child as soon as he can drink. Give the child approximately 5 ml of ORS solution per kilogram of body weight per hour.

Reassess dehydration and choose the appropriate treatment plan

Assess the signs of dehydration in an infant after 6 hours and a child after 3 hours. Classify dehydration. Select the appropriate treatment plan (Plan A, B or C) to continue treatment.

After a child has been fully rehydrated and is classified as NO DEHYDRATION, keep the child at the clinic for 6 more hours if possible. During this time, the mother should give extra fluid according to Plan A. Watch to be sure that the mother can give enough fluid to fully replace all fluid lost while the diarrhoea continues. The child should also be fed. Check the child periodically to make sure that signs of dehydration do not return.

2. If intravenous (IV) treatment is available nearby

You are not able to provide IV treatment at your clinic. However, IV treatment is available at a clinic or hospital nearby (within 30 minutes). Read the Plan C section below that describes this situation.

- Refer URGENTLY to hospital for IV treatment.
- If the child can drink, provide the mother with ORS solution and show her how to give frequent sips during the trip.

Refer the severely dehydrated child immediately to the nearby facility. If the child can drink, show the mother how to give sips of ORS solution to the child. She should encourage her child to drink on the way to the facility.

3. If you are trained to use a nasogastric (NG) tube

You cannot give IV treatment at your clinic and there is no nearby clinic or hospital offering IV treatment. If you are trained to use an NG tube¹, rehydrate the child by giving ORS solution with an NG tube. Read the sections of Plan C below. They describe the steps to rehydrate a child by NG tube.²

- Start rehydration by tube (or mouth) with ORS solution: give 20 ml/kg/hour for 6 hours (total of 120 ml/kg).
- Reassess the child every 1–2 hours:
 - If there is repeated vomiting or increased abdominal distension, give the fluid more slowly.
 - If hydration status is not improving after 3 hours, send the child for IV therapy.
- After 6 hours, reassess the child. Classify dehydration. Then choose the appropriate plan (A, B, or C) to continue treatment.

Note:

- If possible, observe the child at least 6 hours after rehydration to be sure the mother can maintain hydration giving the child ORS solution by mouth.

Some of the terms in this part of Plan C may be new to you. The following explanations will help you understand them.

¹ This annex will not teach you now to use an NG tube to give fluids.

² According to Plan C, the same steps are followed to rehydrate a child by NG tube as by mouth.

- **ABDOMINAL DISTENSION** means the abdomen has increased in size. The skin is stretched.
- **HYDRATION STATUS** refers to whether the child is normally hydrated or dehydrated and the extent of dehydration. A child classified as **NO DEHYDRATION** has not lost enough fluid to show signs of dehydration. A child classified as **SOME DEHYDRATION** or **SEVERE DEHYDRATION** has less than a normal amount of fluid in the body.

To assess a child's hydration status, refer to the signs on the *ASSESS & CLASSIFY* chart.

Monitor the amount of NG fluid and the child's hydration status

When rehydrating a child who has **SEVERE DEHYDRATION**, you have to monitor the amount of NG fluid that you give over the 6-hour period. You may use a form, similar to the following sample fluid form.

Time (hr)	Volume (ml) Set-up*	Estimated Volume (ml) Remaining	Volume (ml) Received
_____	_____		
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

* For each new bottle/pack, initial or added

The form has 4 columns to record the amount of NG fluid given.

1. **Time:** Record the times that you will check the NG fluid. You will want to monitor the fluid every hour for 6 hours.
2. **Volume set-up:** When you begin to give NG fluids, record the amount of fluid in the container. Each time you replace the NG fluid container, record the amount on the appropriate line on the form at the time of replacement.
3. **Estimated Volume Remaining:** Check the IV fluid remaining in the container at the times listed. The remaining volume cannot be read precisely. Estimate it to the nearest 10 ml (for example—220 ml, 230 ml, 240 ml, etc). Record the estimated amount on the form.
4. **Volume received:** Calculate the amount of NG fluid received by the child at the times listed. To calculate, subtract the “Volume remaining” amount from the “Volume set-up” amount. The answer is the amount of NG fluid the child has received up to the time you are checking. Record that amount on the form.

It is helpful to mark the container with a pen or tape to show the level that should be reached at a certain time. For example, mark the desired level to reach after the first 30 or 60 minutes, each hour, or at the end of 3 or 6 hours. This will help you adjust the rate of the drip correctly. Regulate the number of drops per minute to give the correct amount of fluid per hour.

Example

The sample form below shows the amounts of NG fluid that Sa received during the 6 hours he was treated at the clinic. The health worker gave him 200 ml of ORS solution by NG tube (that is, 20 ml x 10 kg) beginning at 11:00 am.

SAMPLE FLUID FORM

Time (hr)	Volume (ml) Set-up*	Estimated Volume (ml) Remaining	Volume (ml) Received
11:00 am	1000 ml		
12:00 pm	—	800 ml	200 ml
1:00 pm	—	600 ml	400 ml
2:00 pm	—	400 ml	600 ml
3:00 pm	—	200 ml	800 ml
4:00 pm	1000 ml	0 ml	1000 ml
5:00 pm		800 ml	1200 ml

* For each new bottle/pack, initial or added

Reassess the child every 1–2 hours:

- If the child is vomiting repeatedly or has increased abdominal distension, give the NG fluid more slowly.
- If the child's dehydration is not improving after 3 hours, refer the child for IV treatment.
- If the child is improving, continue to give the NG fluid for a total of 6 hours.

Reassess dehydration and choose the appropriate Treatment Plan

After 6 hours of NG fluid, reassess the child for dehydration. Classify dehydration. Select the appropriate treatment plan (Plan A, B or C) to continue treatment.

After a child has been fully rehydrated and is classified as NO DEHYDRATION, keep the child at the clinic for 6 more hours if possible. During this time, the mother should give extra fluid according to Plan A. Watch to be sure that the mother can give enough fluid to fully replace all fluid lost while the diarrhoea continues. The child should also be fed. Check the child periodically to make sure that signs of dehydration do not return.

4. If you can only give Plan C treatment by mouth

You cannot give IV fluids at your clinic. There is no clinic or hospital nearby that can give IV treatment. You are not able to use an NG tube for rehydration.

To learn how to give Plan C treatment by mouth, read the sections of Plan C below. Study the sections carefully.

- Start rehydration by tube (or mouth) with ORS solution: give 20 ml/kg/hour for 6 hours (total of 120 ml/kg).
- Reassess the child every 1–2 hours:
 - If there is repeated vomiting or increased abdominal distension, give the fluid more slowly.
 - If hydration status is not improving after 3 hours, send the child for IV therapy.
- After 6 hours, reassess the child. Classify dehydration. Then choose the appropriate plan (A, B, or C) to continue treatment.

Note:

- If possible, observe the child at least 6 hours after rehydration to be sure the mother can maintain hydration giving the child ORS solution by mouth.

If a child with SEVERE DEHYDRATION comes to your clinic and you cannot give IV or NG treatment, find out if the child is able to drink.

- **If he is able to drink, you can try to rehydrate the child orally.**
- **If the child is not able to drink, you must refer him urgently to the nearest clinic or hospital where IV or NG treatment is available. If this child does not receive fluids, he will die.**

Some of the terms in this part of Plan C may be new to you. The following will help you understand them.

- **ABDOMINAL DISTENSION** means the abdomen has increased in size. The skin is stretched.
- **HYDRATION STATUS** refers to whether the child is normally hydrated or dehydrated and the extent of dehydration. A child classified as **NO DEHYDRATION** has not lost enough fluid to show signs of dehydration. A child classified as **SOME DEHYDRATION** or **SEVERE DEHYDRATION** has less than a normal amount of fluid in the body.

To assess a child's hydration status, refer to the signs on the *ASSESS & CLASSIFY* chart.

Monitor the amount of ORS

If you will rehydrate the child orally, you will have to monitor the amount of ORS solution you give him. Give 20 ml per kilogram of body weight per hour for a 6-hour period. After 6 hours, you will have given the child a total of 120 ml of ORS solution per kilogram of the child's weight.

Reassess the child's hydration status every 1–2 hours.

- If there is repeated vomiting or increasing abdominal distension, give the fluid more slowly.
- If the child's hydration status is not improving after 3 hours, refer the child for IV treatment.

Reassess dehydration and choose the appropriate Treatment Plan

After 6 hours of taking ORS solution by mouth, reassess the child for dehydration. Classify dehydration. Select the appropriate treatment plan (Plan A, B or C), and continue treatment.

After the child is rehydrated, keep the child at the clinic for 6 more hours if possible. During this time, encourage the mother to give extra fluid according to Plan A. Watch to

be sure that the mother can give enough fluid to fully replace all fluid lost while the diarrhoea continues. Check the child periodically to make sure that signs of dehydration do not return.

Remember:

If the child cannot drink, refer the child urgently to the nearest clinic or hospital for IV or NG treatment.

If this child does not receive fluids, he will die.

ANNEX B: SAMPLE CASE RECORDING FORMS

MANAGEMENT OF THE SICK YOUNG INFANT AGE 1 WEEK UP TO 2 MONTHS

Name: _____ Age: _____ Weight: _____ kg Temperature: _____ °C

ASK: What are the infant's problems? _____ Initial visit? _____ Follow-up Visit? _____

ASSESS (Circle all signs present)

CLASSIFY

CHECK FOR POSSIBLE BACTERIAL INFECTION														
Has the infant had convulsions?	- Count the breaths in one minute. _____ breaths per minute Repeat if elevated _____ Fast breathing? - Look for severe chest indrawing. - Look for nasal flaring. - Look and listen for grunting. - Look and feel for bulging fontanelle. - Look for pus draining from the ear. - Look at umbilicus. Is it red or draining pus? Does the redness extend to the skin? - Fever (temperature 37.5°C or feels hot) or low body temperature (below 35.5°C or feels cool). - Look for skin pustules. Are there many or severe pustules? - See if young infant is lethargic or unconscious. - Look at young infant's movements. Less than normal?													
DOES THE YOUNG INFANT HAVE DIARRHOEA?		Yes ____ No ____												
- For how long? _____ Days - Is there blood in the stools?	- Look at the young infant's general condition. Is the infant: Lethargic or unconscious? Restless or irritable? - Look for sunken eyes. - Pinch the skin of the abdomen. Does it go back: Very slowly (longer than 2 seconds)? Slowly?													
THEN CHECK FOR FEEDING PROBLEM OR LOW WEIGHT														
- Is there any difficulty feeding? Yes ____ No ____ - Is the infant breastfed? Yes ____ No ____ - If Yes, how many times in 24 hours? _____ times - Does the infant usually receive any other foods or drinks? Yes ____ No ____ If Yes, how often? - What do you use to feed the child?	Determine weight for age. Low ____ Not Low ____													
If the infant has any difficulty feeding, is feeding less than 8 times in 24 hours, is taking any other food or drinks, or is low weight for age AND has no indications to refer urgently to hospital: ASSESS BREASTFEEDING: - Has the infant breastfed in the previous hour? If infant has not fed in the previous hour, ask the mother to put her infant to the breast. Observe the breastfeed for 4 minutes. - Is the infant able to attach? To check attachment, look for: <table border="0" style="width: 100%;"> <tr> <td>— Chin touching breast</td> <td>Yes ____</td> <td>No ____</td> </tr> <tr> <td>— Mouth wide open</td> <td>Yes ____</td> <td>No ____</td> </tr> <tr> <td>— Lower lip turned outward</td> <td>Yes ____</td> <td>No ____</td> </tr> <tr> <td>— More areola above than below the mouth</td> <td>Yes ____</td> <td>No ____</td> </tr> </table> <i>no attachment at all not well attached good attachment</i> - Is the infant suckling effectively (that is, slow deep sucks, sometimes pausing)? <i>not suckling at all not suckling effectively suckling effectively</i> - Look for ulcers or white patches in the mouth (thrush).			— Chin touching breast	Yes ____	No ____	— Mouth wide open	Yes ____	No ____	— Lower lip turned outward	Yes ____	No ____	— More areola above than below the mouth	Yes ____	No ____
— Chin touching breast	Yes ____	No ____												
— Mouth wide open	Yes ____	No ____												
— Lower lip turned outward	Yes ____	No ____												
— More areola above than below the mouth	Yes ____	No ____												
CHECK THE YOUNG INFANT'S IMMUNIZATION STATUS		Return for next immunization on:												
Circle immunizations needed today.														
BCG	DPT1	DPT2												
OPV 0	OPV 1	OPV 2												
	(Date)													

ASSESS OTHER PROBLEMS:

TREAT

[illegible]

MANAGEMENT OF THE SICK CHILD AGE 2 MONTHS UP TO 5 YEARS

Name: _____ Age: _____ Weight: _____ kg Temperature: _____ °C

ASK: What are the child's problems? _____ Initial visit? _____ Follow-up Visit? _____

ASSESS (Circle all signs present)

CLASSIFY

CHECK FOR GENERAL DANGER SIGNS NOT ABLE TO DRINK OR BREASTFEED LETHARGIC OR UNCONSCIOUS VOMITS EVERYTHING CONVULSIONS	General danger signs present? Yes ____ No ____ Remember to use danger sign when selecting classifications
DOES THE CHILD HAVE COUGH OR DIFFICULT BREATHING? Yes ____ No ____ ● For how long? ____ Days ● Count the breaths in one minute. _____ breaths per minute. Fast breathing? ● Look for chest indrawing. ● Look and listen for stridor.	
DOES THE CHILD HAVE DIARRHOEA? Yes ____ No ____ ● For how long? ____ Days ● Is there blood in the stools? ● Look at the child's general condition. Is the child: Lethargic or unconscious? Restless or irritable? ● Look for sunken eyes. ● Offer the child fluid. Is the child: Not able to drink or drinking poorly? Drinking eagerly, thirsty? ● Pinch the skin of the abdomen. Does it go back: Very slowly (longer than 2 seconds)? Slowly?	
DOES THE CHILD HAVE FEVER? (by history/feels hot/temperature 37.5°C or above) Yes ____ No ____ Decide Malaria Risk: High Low ● For how long? ____ Days ● If more than 7 days, has fever been present every day? ● Has child had measles within the last three months? ● Look or feel for stiff neck. ● Look for runny nose. Look for signs of MEASLES: ● Generalized rash and ● One of these: cough, runny nose, or red eyes.	
If the child has measles now or within the last 3 months: ● Look for mouth ulcers. If Yes, are they deep and extensive? ● Look for pus draining from the eye. ● Look for clouding of the cornea.	
DOES THE CHILD HAVE AN EAR PROBLEM? Yes ____ No ____ ● Is there ear pain? ● Is there ear discharge? If Yes, for how long? ____ Days ● Look for pus draining from the ear. ● Feel for tender swelling behind the ear.	
THEN CHECK FOR MALNUTRITION AND ANAEMIA ● Look for visible severe wasting. ● Look for palmar pallor. Severe palmar pallor? Some palmar pallor? ● Look for oedema of both feet. ● Determine weight for age. Very Low ____ Not Very Low ____	
CHECK THE CHILD'S IMMUNIZATION STATUS Circle immunizations needed today. BCG DPT1 DPT2 DPT3 OPV 0 OPV 1 OPV 2 OPV 3 Measles	Return for next immunization on: (Date)
ASSESS CHILD'S FEEDING if child has ANAEMIA OR VERY LOW WEIGHT or is less than 2 years old ● Do you breastfeed your child? Yes ____ No ____ If Yes, how many times in 24 hours? ____ times. Do you breastfeed during the night? Yes ____ No ____ ● Does the child take any other food or fluids? Yes ____ No ____ If Yes, what food or fluids? _____ _____ How many times per day? ____ times. What do you use to feed the child? _____ If very low weight for age: How large are servings? _____ Does the child receive how own serving? ____ Who feeds the child and how? _____ ● During the illness, has the child's feeding changed? Yes ____ No ____ If Yes, how? _____	FEEDING PROBLEMS

ASSESS OTHER PROBLEMS:

TREAT

Remember to refer any child who has a danger sign and no other severe classification.

Return for follow-up in:

Advise mother when to return immediately.

Give any immunizations needed today:

Feeding advice:

ANNEX C: EXAMPLE MOTHER'S CARD

WHEN TO RETURN IMMEDIATELY

BRING ANY SICK CHILD, IF:



► Not able to drink or breastfeed



► becomes sicker



► develops a fever

BRING CHILD WITH COUGH, IF:



► Difficult breathing



► Fast breathing

BRING CHILD WITH DIARRHOEA, IF:



► Blood in stool

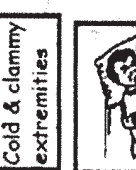


► Drinking poorly

Bring child with possible DHF or fever DHF unlikely, if:



► Epigastric pain or restlessness



► Cold & clammy extremities



► Any signs of bleeding

BRING YOUNG INFANT (less than 2 months old), IF:



► Breastfeeding poorly



► Any of the above signs

Name : _____ M/F. Date of birth: _____

Address : _____

ALWAYS BRING THIS CARD WITH YOU TO THE CLINIC

FLUIDS

► FOR ANY SICK CHILD:

• Breastfeed frequently

• Increase fluid.

Give; soup, rice water, meat broth, clean boiled water.

► FOR CHILD WITH DIARRHOEA:

Give more fluid can be lifesaving !

• Give these extra fluids, as much as the child will takes :

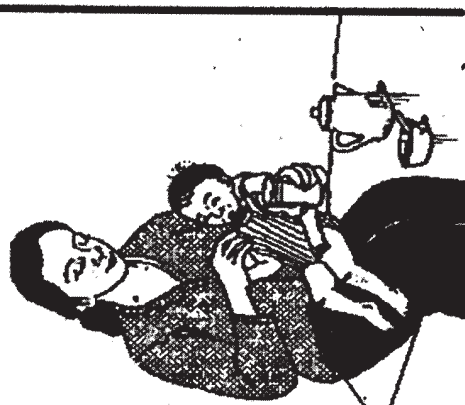
☐ ORS

☐ Food base fluids, such as soup, rice water, meat broth

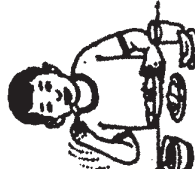
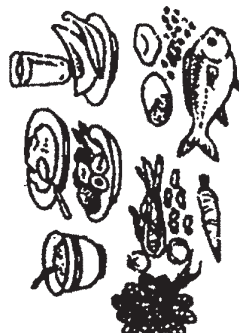
☐ Clean boiled water

• Breastfeed more frequently and longer at each feeding

• Continue giving extra fluids until diarrhoea stops



► Feeding Recommendations During Sickness and Health

Up to 4 Months of Age	4 Months up to 6 months	6 Months up to 12 months	12 Months up to 2 Years	2 Years and Older												
																
- Breastfeed as often as the child wants, day and night, at least 8 times in 24 hours. - Do not give other foods or fluids.	- Breastfeed as often as the child wants, day and night, at least 8 times in 24 hours. - Give complementary food (see the list below) after breastfeeding. Mashed rice-porridge added with egg-yolk/ chicken/ fish/ tempe/ tahu/ beet/ carrot/ spinach/ green beans/ oil/ coconut milk. - Give these foods 2 times per day, each time 2 tablespoons.	- Breastfeed as often as the child wants. - Give adequate amount of rice porridge with egg/chicken/fish/meat, tempe/tahu/carrot/spinach/green beans/oil/coconut milk - Give this food 3 times per day, each time as follows:<table><tr><td>age 6 months</td><td>: 6 table spoons</td></tr><tr><td>7 months</td><td>: 7 "</td></tr><tr><td>8 months</td><td>: 8 "</td></tr><tr><td>9 months</td><td>: 9 "</td></tr><tr><td>10 months</td><td>: 10 "</td></tr><tr><td>11 months</td><td>: 11 "</td></tr></table> - PLUS: - Feed the baby nutritious snacks such as : green beans, porridge, banana, biscuit, nagasari etc. 2 times per day, each time between meals.	age 6 months	: 6 table spoons	7 months	: 7 "	8 months	: 8 "	9 months	: 9 "	10 months	: 10 "	11 months	: 11 "	- Breastfeed as often as the child wants. - Give adequate servings of steamed rice with egg/ chicken/ fish/ meat/ tempe/tahu/ carrot/ spinach/ green beans/ oil/ coconut milk. - Give this food at least 3 times per day - Give also snacks 2 times per day between meals such as : green beans porridge, banana, biscuits, nagasari etc.	- Give adequate amount of family foods in 3 meals each day, consisting of rice, side dishes, vegetables and fruits. - Also, twice daily, give nutritious foods between meals, such as : - green beans porridge - biscuits - nagasari
age 6 months	: 6 table spoons															
7 months	: 7 "															
8 months	: 8 "															
9 months	: 9 "															
10 months	: 10 "															
11 months	: 11 "															
																

- Wash your hands before preparing the child's food
- Use good and safe food ingredients, only use clean cooking utensils and prepare the food properly

Feeding Recommendations For a Child Who Has PERSISTENT DIARRHOEA

- If s/s of breastfeeding, give more frequent, longer breastfeeds, day and night.
- If taking other milk:
 - replace with increased breastfeeding OR
 - replace half the milk with rice porridge and tempe
 - DO NOT USE CONDENSED OR EVAPORATED MILK.
- For other foods, follow feeding recommendations for the child's age.

Glossary

Abscess: a collection of pus

Sterile abscess: an abscess that contains no bacteria

Abdomen: the area of the body containing the stomach and bowel

Abdominal: in the abdomen

Active feeding: encouraging a child to eat, for example, by sitting with him and helping to get the spoon to his mouth

Active neurological disease of the central nervous system: epilepsy and other current diseases of the brain or spinal cord. This does not include permanent, old neurological problems from cerebral palsy, polio, or injuries.

AIDS: Acquired Immune Deficiency Syndrome, caused by infection with the Human Immunodeficiency Virus (HIV). AIDS is the final and most severe phase of HIV infection. The immune system works poorly, and the patient may have various symptoms and diseases (such as diarrhoea, fever, wasting, pneumonia).

Amoebiasis: amoebic dysentery; dysentery caused by the amoeba *E. histolytica*

Allergies: problems such as sneezing, a rash, or difficult breathing that affect certain people when specific things are breathed in, eaten, injected, or touched

Antidiarrhoeal drugs: drugs that are claimed to stop or decrease diarrhoea, such as antimotility drugs. These drugs are not useful for children with diarrhoea. Some are dangerous.

Antiemetics: drugs to control vomiting

Antifolate drugs: drugs that act against folate. Both cotrimoxazole (trimethoprim-sulfamethoxazole) and the antimalarial sulfadoxine-pyramethamine (Fansidar) are antifolate drugs.

Antimotility drugs: drugs that slow the movement of contents through the bowel by reducing its muscular activity

Appetite: the desire to eat

Areola: the dark circle of skin around the nipple of the breast

Aspiration: inhaling (breathing in) fluids

Assess: to consider the relevant information and make a judgement. As used in this course, to examine the child and identify the signs of illness.

Axillary temperature: temperature measured in the armpit

BCG: an immunization to prevent tuberculosis, given at birth. The initials stand for Bacille Calmette-Guerin.

Bowel: intestine

Breast cancer: malignant tumor that starts in the breast

Breastmilk substitute: Formula or milk given instead of or in addition to breastmilk. An example is cow's milk made as follows: Mix $\frac{1}{2}$ cup (100 ml) boiled whole cow's milk with $\frac{1}{4}$ cup (50 ml) boiled water and 2 level teaspoons (10 grams) of sugar.

Cerebral malaria: falciparum malaria affecting the brain

Checking questions: questions intended to find out what someone understands and what needs further explanation. After teaching a mother about feeding, a health worker might ask the checking question, “What foods will you feed your child?”

Chest indrawing: when the lower chest wall (lower ribs) goes in when a child breathes in. In a child age 2 months up to 5 years, if chest indrawing is clearly visible and present all the time during an examination, it is sign of SEVERE PNEUMONIA OR VERY SEVERE DISEASE.

Severe chest indrawing: chest indrawing that is very deep and easy to see. In a young infant, mild chest indrawing is normal, but severe chest indrawing is a sign of serious illness.

Chronic: lasting a long time or recurring frequently

Classify: as used in this course, to select a category of illness and severity (called a classification) based on a child’s signs and symptoms.

Clinic: as used in this course, any first-level outpatient health facility such as dispensary, rural health post, health centre, or the outpatient department of a hospital

Communication skills: as used in this course, skills used in teaching and counselling with mothers, including: ASK AND LISTEN, PRAISE, ADVISE, AND CHECK UNDERSTANDING

Complementary foods: foods given in addition to breastmilk, starting when a child is 4-6 months of age. By age 6 months, all children should be receiving a nutritious, thick complementary food, such as cereal mixed with oil and bits of meat, vegetables, or fish. Complementary foods are sometimes called “weaning foods.”

Confidence: a feeling of being able to succeed

Contraindication: a situation or condition in which a certain treatment, procedure or drug should not be used

Corneal rupture: bursting of the cornea, that is, the clear outer layer of the eye

Counsel: as used in this module, to teach or advise a mother as part of a discussion which includes: asking questions, listening to the mother’s answers, praising and/or giving relevant advice, helping to solve problems, and checking understanding

Counselling: the process of teaching or advising as described above

Deficiency: a lack or shortage. Vitamin A deficiency is a shortage of vitamin A in the body.

Dehydration: loss of a large amount of water and salts from the body

Diagnostic testing: special testing, such as laboratory tests or X rays, to determine the type or cause of illness

Digest: to process food so it can be absorbed and used in the body

Digital watch: a watch that shows the time in digits (numerals) instead of with moving hands

Disease: as used in this course, a specific illness or group of illnesses, classified on the basis of signs and symptoms, for example, “VERY SEVERE FEBRILE DISEASE.” This classification includes several illnesses such as meningitis, cerebral malaria, and septicaemia.

DPT: immunization to prevent diphtheria, pertussis (whooping cough), and tetanus. For full protection a child needs 3 injections: at 6 weeks, 10 weeks, and 14 weeks.

Energy-rich: full of ingredients that give energy (or calories), such as starches or oil

Engorgement: a condition in which a mother’s breasts are swollen, hard and painful because they are too full of milk

Episodes: occurrences of a disease

Diarrhoeal episodes: occurrences of diarrhoea

Essential: necessary. Essential vitamins and minerals (such as vitamins and iron) are those necessary for good health.

Essential fatty acids: fats that are necessary for a baby's growing eyes and brain. These fatty acids are not present in cow's milk or most brands of formula.

Exclusive breastfeeding: giving a child only breastmilk and no additional food, water, or other fluids (with the exception of medicines and vitamins, if needed)

Expertise: a high level of skill in a particular area

Falciparum malaria: malaria caused by the parasite *Plasmodium falciparum*

Family foods: foods ordinarily eaten by the family

Febrile: having fever

Feeding assessment: the process of asking questions to find out about a child's usual feeding and feeding during illness. (Appropriate questions are listed on the COUNSEL chart.)

Feeding bottle: a bottle with a nipple or teat that a child sucks on. Feeding bottles should not be used.

Feeding problems: differences between a child's actual feeding and feeding recommendations listed on the COUNSEL chart, and other problems such as difficulty breastfeeding, use of a feeding bottle, lack of active feeding, or not feeding well during illness

Femoral artery: the main artery to the leg. Its pulsation can be felt in the groin (upper inner thigh).

Femoral vein: the main vein from the leg. It is located just medial to the femoral artery (that is, towards the middle of the body from the femoral artery).

Fever: as used in this course, fever includes:

- a history of fever (as reported by the mother)
- feeling hot to the touch
- an axillary temperature of 37.5 °C (99.5 °F) or higher, or a rectal temperature of 38 °C (100.4 °F) or higher.

First-level health facility: a facility such as a health centre, clinic, rural health post, dispensary, or outpatient department of a hospital, which is considered the first facility within the health system where people seek care. In this course, the term clinic is used for any first-level health facility.

Folate: folic acid, a vitamin used in treatment of nutritional anaemia

Follow-up visit: a return visit requested by the health worker to see if treatment is working or if further treatment or referral is needed

Fontanelle: the soft spot on top of a young infant's head, where the bones of the head have not come together

Full-term: word used to describe a baby born after 37 weeks of pregnancy

Glucose: a sugar used in oral rehydration salts and in IV fluids

Gruel: a food made by boiling cereal meal or legumes in milk or water. Gruel may be made thick like a porridge or thin like a drink. For complementary feeding, gruel should be made thick.

Grunting: soft, short sounds that a young infant makes when breathing out. Grunting occurs when a young infant is having trouble breathing.

Guilty: a feeling of having done wrong

Haemoglobin: a protein containing iron that carries oxygen and makes the blood red

Hepatitis B virus: one of several viruses that cause hepatitis; this virus also causes liver cancer. This virus is spread easily by blood, so needles and syringes must be sterile.

HIV: Human Immunodeficiency Virus. HIV is the virus that causes AIDS.

Hookworm: a small worm that may live as a parasite in a person's intestine and suck blood. This blood loss may lead to anaemia.

Hospital: as used in this course, any health facility with inpatient beds, supplies, and expertise to treat a very sick child

Hygienically: using clean utensils and clean hands, avoiding germs

Hypernatraemia: too much sodium in the blood

Hypothermia: low body temperature (below 35.5 °C axillary or 36 °C rectal temperature)

Hypoxia: a condition in which too little oxygen is reaching the organs of the body

Illness: sickness. As described in this course, the signs and symptoms of illness need to be assessed and classified in order to select treatment.

Immune suppression: weakening of the immune system so that the body has little resistance to disease

Immune system: the system that helps the body resist disease by producing antibodies or special cells to fight disease-causing agents

Immunization status: a comparison of a child's past immunizations with the recommended immunization schedule. Immunization status describes whether or not a child has received all of the immunizations recommended for his age, and, if not, what immunizations are needed now.

Incompetent: lacking the ability or skill to do something

Infant: as used in this course, a baby up to age 12 months

Young infant: as used in this course, a baby age 1 week up to 2 months

Infant feeding formulas: concentrated milk or soy products (to be combined with water) sold as a substitute for breastmilk.

Initial visit: the first visit to a health worker for an episode of an illness or problem

Inpatient: a patient who stays at a health facility and receives a bed and food as well as treatment

Integrated: combined

Integrated case management process: a process for treating patients that includes consideration of all of their symptoms

Intramuscular (IM) injection: an injection (shot) put into a muscle, usually of the thigh

Intravenous (IV) infusion: continuous slow introduction of a fluid into a vein

Intravenous (IV) injection: an injection (shot) put directly into a vein

Jaundiced: having a yellow color in the eyes and skin

Koplik spots: spots that occur in the mouth inside the cheek during the early stages of measles. They are small, irregular, bright red spots with a white spot in the center. They do not interfere with drinking or eating and do not need treatment.

Kwashiorkor: a type of protein-energy malnutrition due to lack of protein in the diet. A child with kwashiorkor has oedema, which may cause his limbs to appear puffy. The child may have sparse hair and dry scaly skin..

Lactose: a sugar present in milk

Local: present in the nearby geographic area. For example, local foods are those found in the area. (See “local infections” below for another meaning of “local.”)

Local infections: infections located only in a specific place on the body, for example, in the eye or in the mouth

Low blood sugar: too little sugar in the blood, also called hypoglycaemia.

Low birthweight: low weight at birth, due either to poor growth in the womb or to prematurity (being born early). Children less than 2500 grams have low birthweight.

Malignant: tending to spread and result in death

Marasmus: a type of protein-energy malnutrition due to long-term lack of calories and protein. A child with marasmus appears to be just “skin and bones.”

Mastoid: the skull bone behind the ear

Measles complications: problems or infections that occur during or after measles. Some examples of measles complications are: diarrhoea, pneumonia, stridor, mouth ulcers, ear infection, and eye infection. A less common complication is encephalitis, an inflammation of the brain.

Meningitis: a dangerous infection in which the spinal fluid and the membranes surrounding the brain and spinal cord become infected

Midwife: a health care worker who assists women in childbirth and may also provide other health care

Nasogastric (NG) tube: a tube inserted through a patient’s nose to his stomach. An NG tube may be used to give ORS solution to severely dehydrated patients when IV therapy is not available, or to feed a severely malnourished child who cannot eat.

Nutrient: a substance in food that helps one grow and be healthy, such as protein, minerals, and vitamins

Nutrient-rich: full of the essential nutrients. These include protein as well as vitamins and minerals.

Nutritional status: the degree to which a child shows or does not show certain signs of malnutrition or anaemia or low weight. In this course, a child’s nutritional status may be classified as: SEVERE MALNUTRITION OR SEVERE ANAEMIA, ANAEMIA OR VERY LOW WEIGHT, or NO ANAEMIA AND NOT VERY LOW WEIGHT.

Oedema: swelling from excess fluid under the skin. Oedema usually occurs in the lower legs and feet, sometimes elsewhere.

Opportunistic infections: infections caused by microorganisms which the body’s immune system is normally able to fight off. When the immune system is weakened, as in AIDS, opportunistic infections can take hold. For example, in a healthy person, there are organisms in the mouth which do not normally cause infection; however, in a person with a weakened immune system, these same organisms may cause oral thrush.

Oral Rehydration Salts (ORS): a mixture of glucose and salts conforming to the WHO recommended formula (in grams per litre): sodium chloride 3.5; trisodium citrate, dihydrate 2.9, or sodium bicarbonate 2.5; potassium chloride 1.5; and glucose 20.0.

OPV: oral polio vaccine. To prevent polio, it is given in 4 doses: at birth, 6 weeks, 10 weeks, and 14 weeks.

- Outpatient:** a patient who does not stay overnight at a health facility
- Ovarian cancer:** malignant tumors starting in the ovaries (the female sex glands in which eggs are formed)
- Overwhelmed:** feeling as though there is too much to do or remember
- Parasite:** an organism living in or on another organism and causing it harm
- Pathogen:** an organism or microorganism that causes disease
- Persist:** to remain or endure
- Practical:** possible to do with the resources and time available
- Pre-referral:** before referral to a hospital
- Premature:** born early, before 37 weeks of pregnancy
- Protein:** a substance in food made up of amino acids needed for adequate growth. Meat, fish, eggs, milk, and beans are examples of foods containing protein.
- Protein-energy malnutrition:** a condition caused by lack of enough protein or energy in the diet, or by frequent illness.
- Pulses:** legumes, such as peas, beans, or lentils
- Pustule:** a reddish bump on the skin containing pus
- Radial pulse:** the pulse felt over the radial artery, which is the main blood vessel at the wrist on the outside of the thumb
- Reassessment:** as used in this course, to examine the child again for signs of specific illness to see if the child is improving
- Full reassessment:** to do the entire assessment process on the *ASSESS & CLASSIFY* chart again to see if there has been improvement and also to assess and classify any new problems
- Recommendations:** advice, instructions that should be followed
- Recurrent convulsions:** spasms or fits that occur repeatedly
- Reduce, reduction:** decrease
- Referral:** as used in this course, sending a patient for further assessment and care at a hospital
- Relactation:** starting breastfeeding again and producing breastmilk after stopping
- Respiratory distress:** discomfort from not getting enough air into the lungs
- Semi-solid food:** food that is part solid and part liquid. A soft, wet food such as gruel or porridge is semi-solid.
- Septicaemia:** an infection of the blood, also called “sepsis” in this course
- Severe classification:** as used in this course, a very serious illness requiring urgent attention and usually referral or admission for inpatient care. Severe classifications are listed in pink-colored rows on the *ASSESS & CLASSIFY* chart
- Shock:** a dangerous condition with severe weakness, lethargy, or unconsciousness, cold extremities, and fast, weak pulse. It is caused by diarrhoea with very severe dehydration, haemorrhage, burns, or sepsis.
- Signs:** as used in this course, physical evidence of a health problem which the health worker observes by looking, listening, or feeling. Examples of signs include: fast breathing, chest indrawing, sunken eyes, stiff neck, pus draining from the ear, etc.
- Stable:** staying the same rather than getting worse

Symptoms: as used in this course, health problems reported by the mother such as cough, diarrhoea, or ear pain

Main symptoms: as used in this course, those symptoms which the health worker should ask the mother about when assessing the child. The four main symptoms listed on the *ASSESS & CLASSIFY* chart are: cough or difficult breathing, diarrhoea, fever, and ear problem.

Thrush: ulcers or white patches on the inside of the mouth and tongue, caused by a yeast infection

Trophozoites: stage of a protozoan organism such as *Giardia lamblia* or *E. histolytica*; the stage which causes tissue damage

Ulcer: a painful open sore

Mouth ulcers: sores on the inside of the mouth and lips or on the tongue. These may occur with measles and may be red or have white coating on them. They make it difficult to eat or drink.

Upright: vertical (standing up)

Semi-upright: partly upright, leaning

Urgent: requiring immediate attention, important to save a child's life

Urgent referral: sending a patient immediately for further care at a hospital

Uterus: womb

Vulnerable: endangered, likely to become ill

Weaning foods: another term for complementary foods, given in addition to breastmilk starting at 4–6 months of age

Whipworm: a small worm that may live as a parasite in a person's intestine and suck blood. This blood loss may lead to anaemia and diarrhoea.